



SAKRA WORLD HOSPITAL PATIENT SAFETY AND QUALITY HAND BOOK

DEPARTMENT OF QUALITY & SYSTEMS



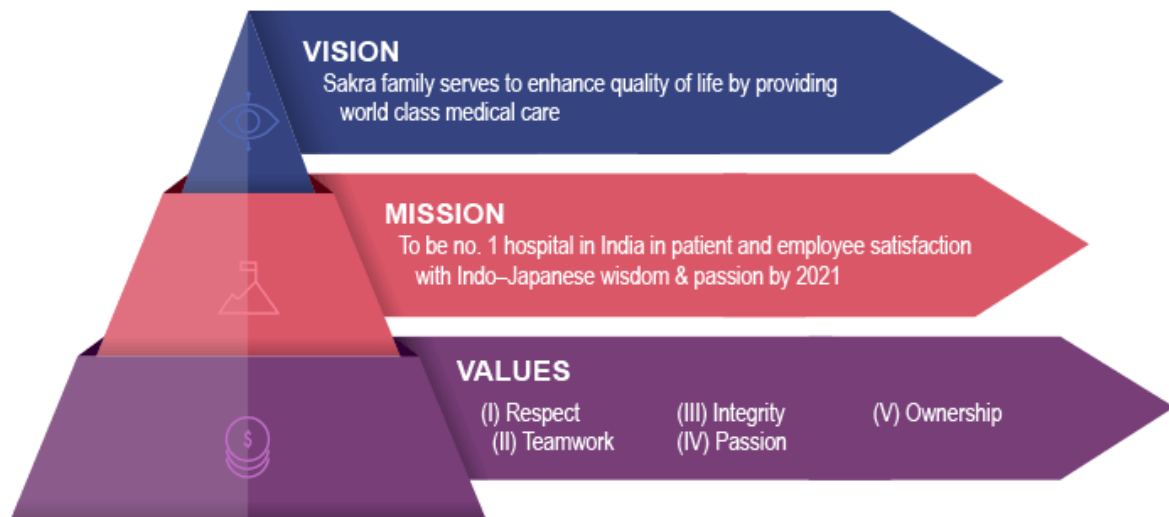
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INTRODUCTION

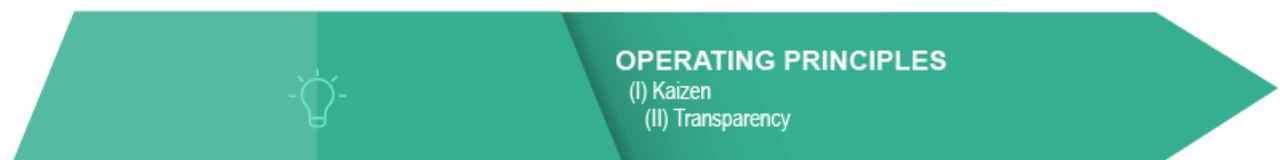
About Sakra World Hospital

- Sakra World Hospital is committed to the advanced medical care that enhances the value of human life
- Sakra combines synergies of these large entities to provide-cutting-edge medical technologies and herald a change in current healthcare systems and processes
- Sakra is dedicated to ensuring good health of the community
- This flagship 350-bedded multi-super specialty hospital situated in Bangalore was launched in February, 2014

VISION & MISSION



OPERATING PRINCIPLE



CORE VALUES



SAKRA BRAND

The name Sakra is derived from two words that have significant reflection of our vision and mission. One of them is a Japanese word 'Sakura', the name that is given to cherry blossom. It has a beautiful and symbolic representation of beauty, combined with proactive energy, hope and good health. It is also the national flower of Japan. The other word is 'Sacrum', which happens to be the last bone of the vertebral column and the one that supports and plays an important part in maintaining the structure and well - being of the human body.



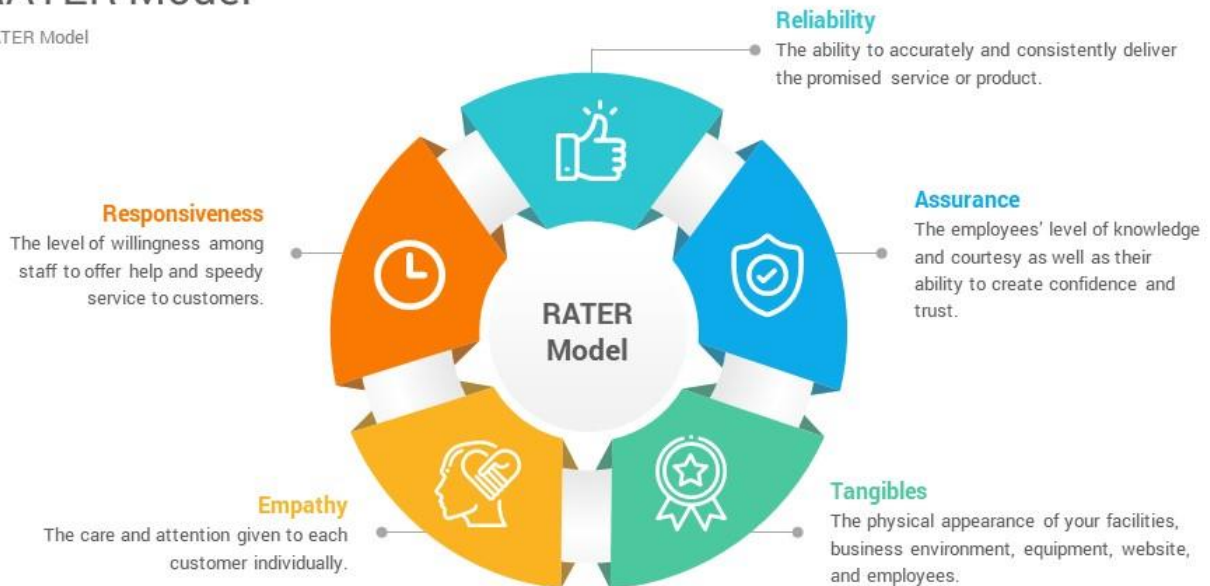
OVERVIEW OF MANAGEMENT



SERVICE STANDARDS

RATER Model

RATER Model



- **Reliability**
 - To provide promised service in an accurate, dependable and consistent manner,
 - with minimal variation
- **Responsiveness**
 - Willingness & Timeliness of employees in providing service.
- **Assurance**
 - Positive declaration intended to give confidence
- **Empathy**
 - Staff to be caring and understanding, and provide individualized attention
- **Tangibles**
 - Refers to aesthetics & appearance of physical facilities, building and equipment.

QUALITY POLICY

To pursue global standards of excellence in all our endeavors and to remain accountable and thrive for continuous improvement to provide patient care with patient centricity, safety, equality, timely and efficiency.

QUALITY OBJECTIVE



QUALITY ASSURANCE

As part of quality management, quality assurance of the following critical areas and services are done on yearly basis based on resource & process management, patient & staff safety, documentation.

Quality assurance of critical areas

OT, ICU, Radiology, Laboratory, Blood Bank

Quality assurance of services

Infection Control, House Keeping, CSSD, Security, Pharmacy, MRD

SAKRA PATIENT SAFETY GOALS:



Identify patient correctly

- Name and UHID are identifiers used to identify patients



Improve effective communication

- Communication during patient handover
- Verbal orders policy to ensure safe medication
- Communication with patient and family in special situation



Improve safety of high-alert medication

- Safe storage of high-alert medication
- Labelling of LASA medication
- Handling of narcotics and chemotherapeutic drugs
- Administration and monitoring of concentrated electrolyte solutions



Ensure safe surgery

- Pre-operative checklist
- Surgical safety checklist
- Site marking



Reduce the risk of healthcare associated infections

- Adherence to standard precautions
- Hand hygiene
- Transmission-based precautions
- Single use of injection devices
- Safe injection and infusion practices



Reduce the risk of patient harm resulting from falls

- Assessment of fall risk
- Monitoring and prevention of fall



Avoiding catheter and tubing misconnections

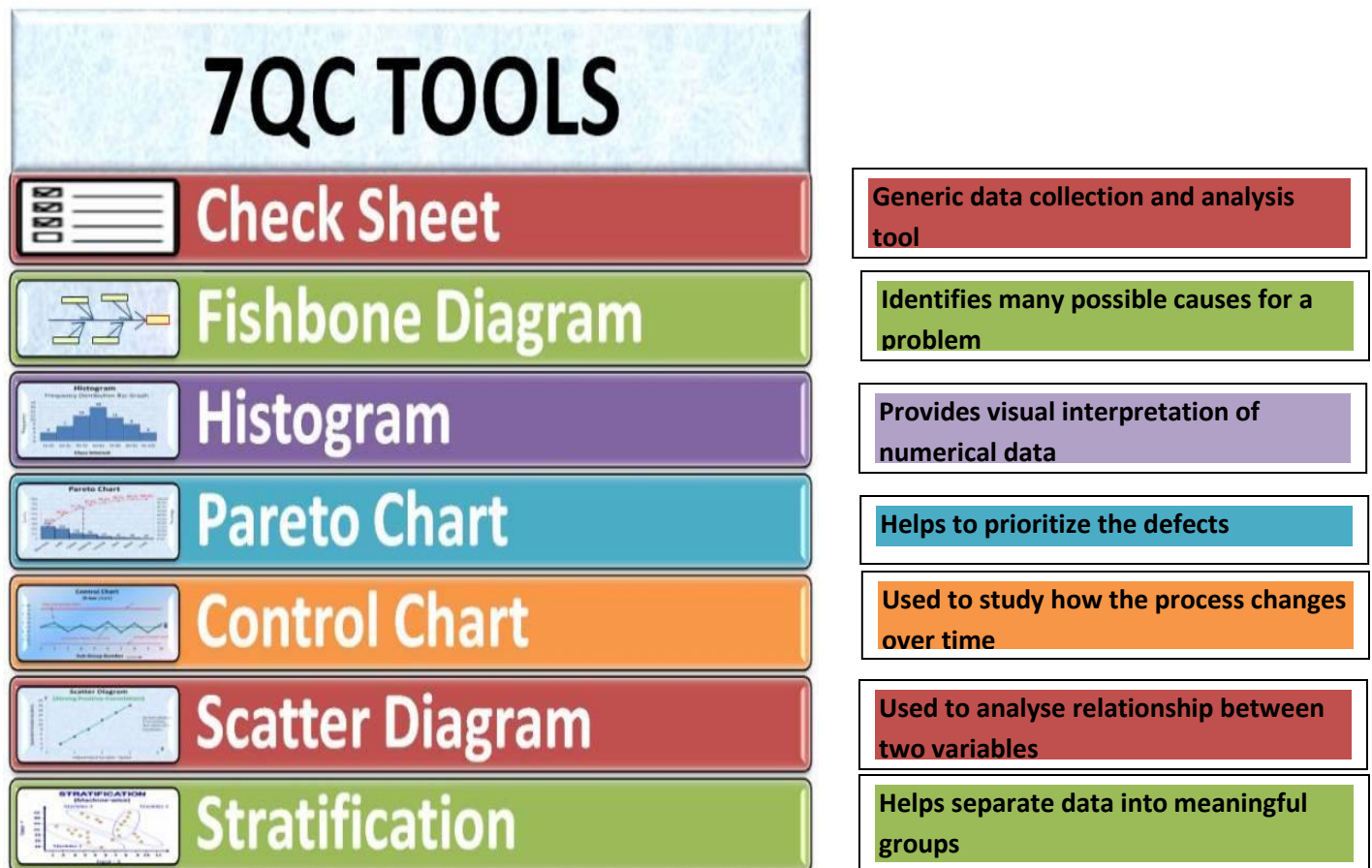
- Standard color system is used to identify the target /intended route of administration



Assuring medication accuracy at transitions in care

- Medication reconciliation
- Appropriate handover to the next provider of care whenever the patient is transferred or discharged

COMMONLY USED ANALYTICAL TOOLS FOR QUALITY IMPROVEMENT PROJECTS



ACCREDITATION & CERTIFICATION OBTAINED BY SAKRA

ACCREDITATION

- National Accreditation Board for Hospitals & Healthcare Providers (NABH)
- Accredited Laboratory Service by National Accreditation Board for Testing and Calibration of Laboratories(NABL)
- Blood Bank Services is Accredited by National Accreditation Board for Hospitals & Healthcare Providers (NABH)

CERTIFICATION

- Nursing Service Excellence Certification

KEY PERFORMANCE INDICATOR (KPI) MONITORED AT SAKRA

32 Key performance Indicators are captured, thereby assuring that the services provided are effective and efficient.

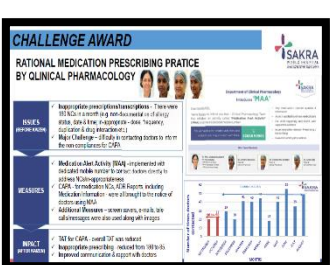
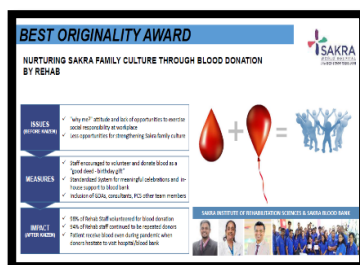
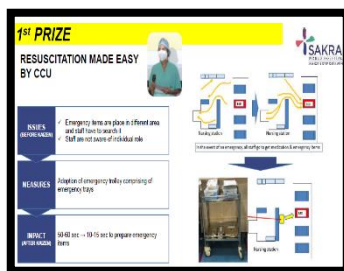
- **Clinical Structure, Process & Outcome**
e. g. Unplanned Return to OT, Medication error.
- **Patient safety**
e. g. Patient fall, Nursing & Doctor handover
- **Managerial Structures, Processes & Outcomes**
e. g. Time taken for Discharge, Nurse patient ratio
- **Infection Control**
e. g. Number of CAUTIs (Catheter Associated Urinary Tract Infections), Hand hygiene

KAIZEN (Continuous Quality Improvement) as quality improvement programme

Kaizen competitions are carried out every year at Sakra, where the staffs showcase the various quality improvement activities carried out by them.

By far, 4 Kaizen competition has been held and 21 quality improvement projects were received from various clinical and non-clinical departments.

KAIZEN COMPETITION 2020



5S (Lean management tool)



We at Sakra believe that every activity should be done in an efficient manner, to reduce any forms of 'muda' (meaning of waste in Japanese) .We have carried out 5S activity across various areas.



5S in Pharmacy



5S in Crash Cart

Quality Circle

QC is a small group of first-line employees in the same work area or doing similar type of work who voluntarily meet regularly for about an hour every week to identify, analyses and resolve work related problems, leading to improvement in their total performance& enrichment of their work life.



CSSD



Security



OPD

HAZARD IDENTIFICATION AND RISK ASSESSMENT (HIRA)

HIRA is carried across entire hospital, which helps to enhance the safety of patients, visitors, and employees; and minimize the financial loss to hospital through risk detection, evaluation, and prevention.

Admission of non critical Covid 19 suspect patients at					Previous Hazard/Risk Events		Existing Control Measures		Residual Risk Rating				Additional Control Required		Responsible person	Target time for completion	Residual Risk Rating(Post Implementation)			
Location/Activity	Hazard/Risk	Groups Exposed	Probability	Severity Impact	H	P	S	Risk Matrix Rating	H	P	S	Risk Matrix Rating	Probability	Severity Impact			H	P	S	Risk Matrix Rating
Attender & visitor management	Cross Infection	All staff & other patients	No	One attendant allowed & two visitors in visiting hours	4	4	1	4	36	No attendant & visitor allowed, patient is allowed to use mobile phones	Security	1-May-20	3	4	1	4	27			
Donning of PPE correctness	Contact /Droplet transmission	All staff & other patients	No	No PPE	4	4	1	4	36	Dedicated PPE for Covid 19. Training given to all staff on donning of PPE	Nursing	25th April	3	4	1	4	27			
Transportation of patient :Vertical	Contact transmission	All staff & other patients	No	Patient is transported in P1 and P2 which is not dedicated for a certain floor. There is no dedicated trolley for shifting suspected patient. Disinfection of lift is not done post transporting.	4	4	1	3	32	Dedicated lift(via) for patient movement. Prohibition of usage of other lifts for Covid suspect patient transportation. A separate hooded trolley is dedicated for Covid suspect patient transportation. Disinfection of lift is done post transporting.	Security/ Nursing	1-May-20	4	4	1	3	32			
Transportation of patient :Horizontal	Contact transmission	Covid ward staff & other patients	No	Patient is transported on a normal bed/stretchers. There was no dedicated route of patient transfer.	4	4	1	3	32	A separate hooded trolley is dedicated for Covid suspect patient transportation. Dedicated route of patient transfer.	Nursing / GDA	1-May-20	3	4	1	3	24			
Sample collection at ward:Throat & Nasal (Swab)	Contact /Droplet transmission	Covid ward staff & other patients	No	Sample collected at ward if required and transported through ANM staff	4	4	1	2	28	Sample is collected at Porta Cabin using full PPE.	Nursing	25th April	4	4	1	2	28			
Sample collection at ward:Blood	Contact /Droplet transmission	Covid ward staff & other patients	No	Nurse collects the sample and sends it to lab through GDA	4	4	1	2	28	Sample is collected wearing full PPE and sent through ANM	Nursing/ ANM	25th April	4	4	1	2	28			
Engineering control	Droplet transmission	All staff & other patients	No	Positive pressure & re circulation of air	4	4	1	4	36	Negative pressure & No recirculation of air	Engineering	20th April	2	4	1	4	18			
Doctor assessment: Caring/handling the patients	Contact /Droplet transmission	Doctors treating suspected Covid 19 patients	No	Doctor might/might not use gloves and mask while initially assessing the patient.	4	3	1	4	40	Doctors compulsorily wear Covid dedicated PPE and does the initial assessment.	Doctors	20th April	4	3	1	4	40			
Nurse assessment: Caring/handling the patients	Contact /Droplet transmission	Nurses caring suspected Covid 19 patients	No	Nurse might/might not use gloves and mask while initially assessing the patient.	4	3	1	4	40	Nurses compulsorily wear Covid dedicated PPE and does the initial assessment.	Nursing	20th April	4	3	1	4	40			
Diagnostic procedure :Bedside	Contact /Droplet transmission	Staff at Covid ward	No	Technician comes to ward without any PPE for any bedside diagnostic procedure. Cassette is not covered with yellow cover	4	3	1	4	40	Technician is provided full PPE before the bedside diagnostic procedure. Equipment disinfection after each procedure. Cassette is taken in yellow cover after use	Radiology/ Nursing	25th April	4	3	1	4	40			
Diagnostic procedure :Other Clinical area:Diagnostic area	Contact /Droplet transmission	Staff at clinical /Diagnostic area	No	Patient is transported to radiology/cardiology/neurology depts for various diagnostic procedures. PPE is not worn by the technicians while handling patients.	4	3	1	4	40	Technician is provided with full PPE before the diagnostic procedure. Equipment disinfection after each procedure is done.	Radiology/ Nursing	25th April	4	3	1	4	40			
Decontamination of room	Contact Transmission	Housekeeping, Nursing staff at Covid ward	No	1 duster is used for 4 beds. Door knobs and handles are not disinfected frequently. Normal cleaning is done after each patient discharge.	4	3	1	3	36	Deep Cleaning is done post discharge of the patient. Door knobs and handles are disinfected. One duster is used for one bed.	HK	1-May-20	4	3	1	3	36			

Structure: Hazard Identification and Risk Assessment in COVID Ward

Sl. No	Process Step	Potential Failure Mode(What can go wrong with this step)	Potential effect of failure(What is the impact on the customer)	Severity(S)	Potential Causes	Occurrence (O)	Current Controls(What are existing controls that prevent either the cause or failure mode prior to leaving the process step?)	Detection(D)	Significance (RPN)	Mitigating Strategies
1	Doctor prescribes medicine for concerned patient	Wrong prescription	Wrong medicines indented	8	Busy schedule of doctor	2	Concerned doctors are informed by the clinical pharmacologists.	7	112	Electronic prescriptions to prevent the wrong manual prescriptions.
		Illegible prescription	Wrong medicines indented	8	Lack of awareness on legible prescriptions	3	Concerned doctors are informed by the clinical pharmacologists.	2	48	Electronic prescriptions to prevent the wrong manual prescriptions.
2	Nurse/Pharmacist indents the medicines	Wrong indenting	Medicines dispensed will go wrong	8	Lack of awareness on brand and generic names	2	Pharmacists are deployed at the nurse station to indent medicines.	8	128	Electronic prescriptions to prevent the wrong indenting.
3	Pharmacist dispenses the medicines for the indent received	Wrong dispensing of medicines	Wrong set of medications reach the ward	5	Double check not done before dispensing.	2	Double check at the time of dispensing in the pharmacy department.	3	30	Electronic prescriptions to prevent the wrong dispensing.
4	Time for administration is checked by the nurse	Nurse might miss the time for administration	Delayed administration or miss of medication administration	7	More patients assigned to nurse.	4	Priority is given to administer the medicine on time over other tasks of the nurse.	9	252	Continuous training to the staff.
5	Medication chart is carried to the patient bedside	Medication chart is left in nurse station.	1. Nurse will not be able to identify the route, dose and frequency of medicines. 2. Nurse might miss documenting the administration.	9	Negligence of the nurse in following the process.	3	Continuous awareness on the medication administration process is given to the nurse.	6	162	Continuous training to the staff.
6	Patient is identified correctly.	Patient identification done incorrectly.	Medication is administered for the wrong patient.	10	Negligence of the nurse in following the process.	3	Continuous awareness on international patient safety goal 1 is emphasized to the nursing staff.	8	240	Automation of patient identification by using barcoded ID band.
7	Prescribed medicine as per chart and availability of physical stock is cross verified	Nurse has to indent the medicine again	Delay in medication administration	3	Improper handover on medications.	2	Pharmacists are deployed at the nurse station to indent medicines.	2	12	Continuous training to the staff.
8	Medication action is explained to the patient.	Patient is unaware of the actions, the medications will have on him.	Consumption of overdose of drugs by the patient.	6	Lack of awareness on medication actions.	4	Continuous awareness on the medication administration process is given to the nurse.	7	168	Continuous training to the staff.
9	Rights of medication administration is followed.	Rights of medication administration not followed	Deviation in medication administration process.	10	Lack of training on rights of medication administration.	3	Continuous awareness on the medication administration process is given to the nurse.	7	210	Continuous training to the staff.
10	Medication administration is carried out on time	Medication administration not done on time	Delay in medication administration	7	Nurse is occupied with many patients.	3	Priority is given to administer the medicine on time over other tasks of the nurse.	8	168	Continuous training to the staff.
11	Documentation is done for the same.	Medication administration not documented.	No record[evidence] of medication administered.	7	Lack of awareness on documentation.	2	Continuous awareness on the medication administration process is given to the nurse.	8	112	EMR to mitigate the documentation errors.

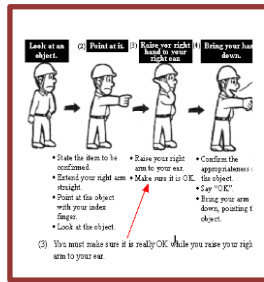
Process: Hazard Identification and Risk Assessment during Medication Administration

KYT (KIKEN YOCHI TRAINING)

Kiken Yochi (hazard prediction) training, is a Japanese activity used to motivate members to recognize and predict hazards. It develops sensitivity to unsafe conditions or hazardous situations. Thus, improving the ability to better recognize.

KYT is a training method whereby, through pre-work meetings or other procedures, workers are given advance warning of these kinds of unsafe conditions or unsafe behavior that are in their immediate sphere of activity, thereby ensuring their own personal safety and the safety of others around them.

At Sakra, KYT is done commonly in patient care areas and diagnostic services:



Laboratory



PMR



Radiology

GREEN HOSPITAL INITIATIVES

WHO has defined green hospital as a hospital, which is responsive to local climate conditions with optimized energy use.

At Sakra, following initiatives have been taken to move towards Green Hospital.



Optimum usage and conservation of water resources



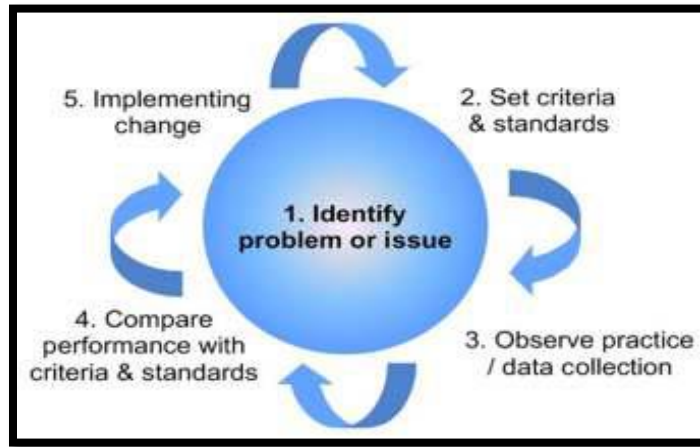
Use of natural light in patient care areas



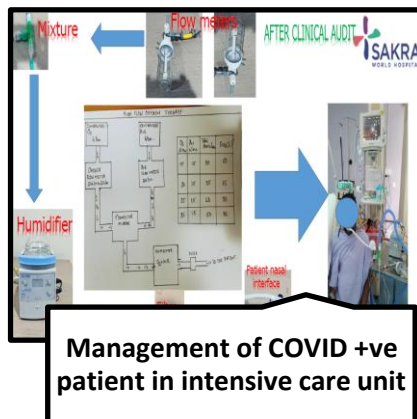
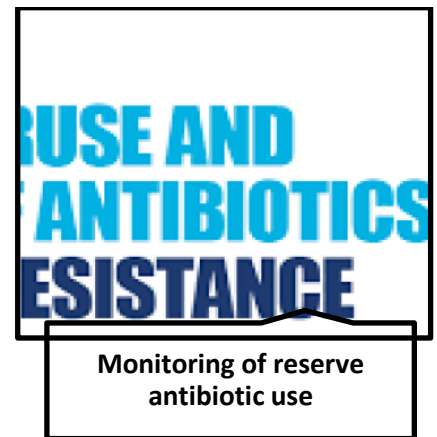
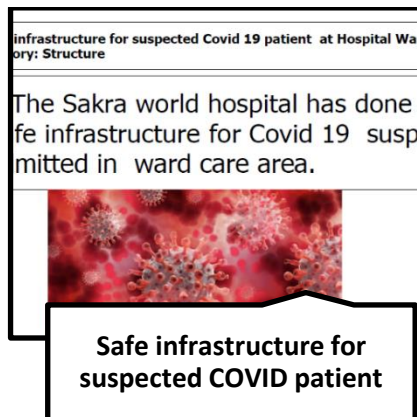
Natural space with 'H' shape design

CLINICAL AUDIT

Clinical audit is carried out to ensure clinical excellence.



Some of the clinical audits carried out are:



CLINICAL PATHWAYS

A clinical pathway is a multidisciplinary management tool based on evidence-based practice for a specific group of patients with a predictable clinical course, in which the different tasks (interventions) by the professionals involved in the patient care are defined, optimized and sequenced either by hour (ED), day (acute care) or visit (homecare). Outcomes are tied to specific interventions.

Coronary Artery Bypass Grafting (CABG) Admission - Treatment Plan				SAKRA WORLD HOSPITAL			
Patient Name: _____		Doctor in charge: _____		Nurse: _____		Patient Signature: _____	
Date	DD/MM/YY	DD/MM/YY	DD/MM/YY	DD/MM/YY	DD/MM/YY	DD/MM/YY	DD/MM/YY
Progress	Day before Surgery (Admission)	Day of Surgery (Before)	Day of Surgery (After)	1st day after Surgery	2nd day after Surgery	3rd day after Surgery	4th day after Surgery
Procedure	1. Admission and history. 2. Wound orientation will be given by nurse. 3. Height, weight, blood pressure, temperature will be examined. 4. Hair removal from surgical site.	1. You will be shifted to OT on a trolley 30 minutes before surgery time. 2. In OT: a. Monitors will be connected. b. 2 injections will be placed in the hand. c. General anesthesia will be given and you will be unconscious.	1. 2-3 hours after surgery, you will wake up in the CTVS ICU with the tube in the mouth connected to the breathing machine. 2. Once you start breathing on your own, the tube will be removed. 3. Other tubes will be there in the nose, on the chest and to pass urine.	1. Urine tube will be clamped and be taken out later. 2. Tube on the chest will be pulled out. 3. Wound dressing will be done. 4. TED stocking will be placed on the legs. 5. Chest binder will be placed. 6. If stable, you will be shifted to the ward.	1. Blood pressure and temperature will be checked regularly by the nurses. 2. Dressing will be done daily. 3. Weight check will be done daily (you may gain 2-5 kg after surgery, which you will lose gradually over the next week).		
Treatment & Medication	1. Stop blood thinners and home medication. 2. Hospital medication will be administered. 3. Insulin injection for sugar control. 4. Blood sugar will be checked 3 times a day.	1. Premedication will be given by the nurse before shifting to OT.	1. All medications will be done by infusion tubes. 2. Additional pain killers will be administered as required.	1. Oral medicine will start. 2. Vital signs will be checked every 2 hours.	Medications will be administered to you by the nurse as needed.		
Examination	1. Blood samples for investigation. 2. X-ray/ECG.	1. Blood samples may be needed.	1. Blood will be sampled as required.	1. Chest X-ray. 2. Blood samples. 3. ECG.	1. Blood samples. 2. ECG.	1. ECHO.	
Activities & Bed Rest Level	1. Level of activity as advised. 2. Physiotherapist will visit to evaluate and educate for cardiac rehabilitation. 3. Preop exercise program will be done.		1. Oxygen mask will be placed after the tube is removed. 2. You need to do the lung exercise/spirometry every hour. 3. You have to lie on the bed in the sitting position for a few hours.	1. Physiotherapist will visit you. 2. You will be made to sit on a chair. 3. You will be made to walk in the ICU. 4. Lung exercise / spirometry to be continued every hour.	1. Write the amount of the liquid consumed in the paper. 2. Measure total amount of urine passed. 3. Do spirometry every hour. 4. Walk as often as possible. 5. Physiotherapist will use you for cardiac rehabilitation. 6. You will do stair climbing at rehabilitation unit at DD/MM (POD 4).		
Diet	1. Diet will be advised by nutritionist. 2. Not to eat or drink 10 hours prior to planned surgery time.		2 hours after removing mouth tube, you can start liquid diet.	Soft diet will be started.	As per your tolerance, you will be put on normal food.		
Cleanliness	Medicated shower to be taken at night.	Medicated shower to be taken in the morning.	Back and body care.		Liquid intake limited to 1.2-1.5 liter / 24 hours (if weight >50 kg).		
Excretion	Can visit the bathroom.		Catheter will be there for urine.		Bed bath will be done.		
Patient and Family Education	1. Physician will explain about surgery and anesthesia. 2. Informal consent will be taken. 3. Nurse will explain procedure & matters that require attention before and after surgery. 3. 2 units of blood to be donated / arranged at the hospital blood bank.	1. Take off glasses, denture, jewellery etc. and hand over to relatives. 2. After surgery you will stay at CTVS ICU. Valuable things should be taken care by relatives.	1. Relatives (only 2) can meet the patient 30 minutes after the surgery in the ICU.		There will be a explanation from nutritionist about diet and meal.		
					There will be a explanation about the therapeutic course from doctor.		

* This is just for information. There is a possibility to change according to the patient's condition. Please acknowledge it.
 * Please do not self medicate without Doctor's instructions.
 * If there is any doubt, please feel free to ask the Doctor and Nurse.

Clinical Pathway: CABG

Valve Replacement Admission - Treatment Plan				SAKRA WORLD HOSPITAL			
Patient Name: _____		Doctor in charge: _____		Nurse: _____		Patient Signature: _____	
Date	DD/MM/YY	DD/MM/YY	DD/MM/YY	DD/MM/YY	DD/MM/YY	DD/MM/YY	DD/MM/YY
Progress	Day before Surgery (Admission)	Day of Surgery (Before)	Day of Surgery (After)	1st day after Surgery	2nd day after Surgery	3rd day after Surgery	4th day after Surgery
Procedure	1. Admission and history. 2. Wound orientation will be given by nurse. 3. Height, weight, blood pressure, temperature will be examined. 4. Hair removal from surgical site.	1. You will be shifted to OT on a trolley 30 minutes before surgery time. 2. In OT: a. Monitors will be connected. b. 2 injections will be placed in the hand. c. General anesthesia will be given and you will be unconscious.	1. 2-3 hours after surgery you will wake up in the CTVS ICU with the tube in the mouth connected to the breathing machine. 2. Once you start breathing on your own, the tube will be removed. 3. Other tubes will be there in the nose, on the chest and to pass urine.	1. Urine tube will be clamped and taken out later. 2. Tube on the chest will be pulled out. 3. Wound dressing will be done. 4. Chest binder will be placed. 5. If stable, you will be shifted to the ward.	1. Blood pressure and temperature will be checked regularly by the nurses. 2. Dressing will be done daily. 3. Weight check will be done daily (you may gain 2-5 kg after surgery, which you will lose gradually over the next week). 4. Pacing wires will be removed.		
Treatment & Medication	1. Stop blood thinners and home medication. 2. Hospital medication will be administered. 3. Insulin injection for sugar control. 4. Blood sugar will be checked 3 times a day.	1. Premedication will be given by the nurse before shifting to OT.	1. All medications will be done by infusion tubes. 2. Additional pain killers will be administered as required.	1. Oral medicine will start. 2. Vital signs will be checked every 2 hours.	Medications will be administered to you by the nurse as needed.		
Examination	1. Blood samples for investigation. 2. X-ray/ECG. 3. Dental Examination.	1. Blood samples may be needed.	1. Blood will be sampled as required.	1. Chest X-ray. 2. Blood samples. 3. ECG.	1. Blood samples. 2. ECG.	1. Blood samples. 2. ECHO.	1. Blood Samples.
Activities & Bed Rest Level	1. Level of activity as advised. 2. Physiotherapist will visit to evaluate and educate for cardiac rehabilitation. 3. Preop exercise program will be done.		1. Oxygen mask will be placed after the tube is removed. 2. You need to do the lung exercise/spirometry every hour. 3. You have to lie on the bed in the sitting position for a few hours.	1. Physiotherapist will visit you. 2. You will be made to sit on a chair. 3. You will be made to walk in the ICU. 4. Lung exercise/spirometry to be continued every hour.	1. Write the amount of the liquid consumed in the paper. 2. Measure total amount of urine passed. 3. Do spirometry every hour. 4. Walk as often as possible. 5. Physiotherapist will use you for cardiac rehabilitation. 6. You will do stair climbing at rehabilitation unit at DD/MM (POD 4).		
Diet	1. Diet will be advised by nutritionist. 2. Not to eat or drink 10 hours prior to planned surgery time.		2 hours after removing mouth tube, you can start liquid diet.	Soft diet will be started.	As per your tolerance, you will be put on normal food.		
Cleanliness	Medicated shower to be taken at night.	Medicated shower to be taken in the morning.	Back and body care.		Liquid intake limited to 1.2-1.5 liter/24 hours (if weight >50 kg).		
Excretion	Can visit the bathroom.		Catheter will be there for urine.		Bed bath will be done.		
Patient and Family Education	1. Physician will explain about surgery and anesthesia. 2. Informal consent will be taken. 3. Nurse will explain procedure & matters that require attention before and after surgery. 3. 2 units of blood to be donated / arranged at the hospital blood bank.	1. Take off glasses, denture, jewellery etc. and hand over to relatives. 2. After surgery you will stay at CTVS ICU. Valuable things should be taken care by relatives.	1. Relatives (only 2) can meet the patient 30 minutes after the surgery in the ICU.		There will be a explanation from nutritionist about diet and meal.		
					There will be a explanation about the therapeutic course from doctor.		

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 * Please do not self medicate without doctor's instruction.
 * If there is any doubt, please feel free to ask doctor and nurse.

Clinical Pathway: Valve Replacement

CAMPAIGN

Various campaigns are organized throughout the year to raise awareness of the issue, to educate the staff about the concerning issue, and to motivate them to speak and act in support of change.

At Sakra, following campaigns are organized:



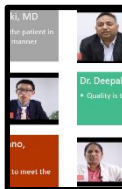
**Patient Safety
Campaign**



**Upkeep of Equipment
Campaign**



**Infection Control
Campaign**



Quality Campaign

NABH FREQUENTLY ASKED QUESTIONS



AAC

ACCESS,ASSESSMENT,CONTINUITY OF CARE



COP

CARE OF PATIENTS



MOM

MANAGEMENT OF MEDICATION



PRE

PATIENT RIGHTS AND EDUCATION



HIC

HOSPITAL INFECTION CONTROL



PSQ

PATIENT SAFETY AND QUALITY IMPROVMENT



ROM

RESPONSIBILITY OF MANAGEMENT



FMS

FACILITY MANAGEMENT AND SAFETY



HRM

HUMAN RESOURCE MANAGEMENT



IMS

INFORMATION MANAGEMENT SYSTEM

ACCESS ASSESSMENT AND CONTINUITY OF CARE (AAC)

1. What are the services being provided at Sakra?



OUR SERVICES

CLINICAL SERVICES

- Anesthesiology
- Cardiac Anaesthesia
- Cardiology
- Cardiothoracic Surgery
- Clinical Hematology
- Cosmetic Surgery
- Critical Care Medicine
- Dental Sciences
- Dermatology
- Diabetology
- Dialysis
- Emergency Medicine
- Endocrinology
- Fetal Medicine
- Gastroenterology (Medical)
- Gastrointestinal Surgery
- General Medicine

- General Surgery
- Hepato-Pancreato-Biliary Surgery
- Interventional Radiology
- Kidney Transplant
- Liver Transplant
- Neonatology
- Nephrology
- Neurology
- Neurosurgery
- Obstetrics and Gynecology
- Oncology
 - Medical Oncology
 - Surgical Oncology
- Ophthalmology
- Orthopaedic Surgery Including Joint

- Replacement
- Otorhinolaryngology (ENT)
- Pain Medicine
- Pediatrics
- Pediatric Surgery
- Physical Medicine and Rehabilitation
- Plastic and Reconstructive Surgery
- Psychiatry
- Reproductive Medicine (Including IVF)
- Respiratory Medicine (Pulmonology)
- Rheumatology
- Spine Surgery
- Sports Medicine and Arthroscopy
- Urology
- Vascular Surgery

LABORATORY SERVICES

- Clinical Bio-chemistry
- Clinical Microbiology and Serology
- Clinical Pathology
- Cytopathology
- Genetics
- Hematology
- Histopathology

TRANSFUSION SERVICES

- Blood Bank and Transfusion services

PROFESSIONS ALLIED TO MEDICINE

- Dietetics
- Clinical Nutrition
- Occupational Therapy
- Psychology
- Speech and Language Therapy

DIAGNOSTIC SERVICES

- 2D Echo
- Audiometry
- Bone Densitometry
- CT Scanning
- DSA Lab
- ECG
- EEG
- Holter Monitoring
- Lithotripsy (ESWL)
- Mammography
- MRI
- PFT
- Sleep Study
- Spirometry
- Tread Mill Test
- Ultrasound
- Uro Dynamic Studies
- X-Ray

SUPPORT SERVICES

- Ambulance
- Mortuary
- Pharmacy

SERVICES NOT AVAILABLE

- Bone Marrow Transplant
- Burns Unit
- Geriatrics
- Heart Transplant
- Nuclear Medicine
- Pancreas Transplant
- Psychiatry Inpatient
- Radiation Oncology

ನಮ್ಮಲ್ಲಿ ಲಭ್ಯವಿರುವ ಸೇವೆಗಳು

ವೈದ್ಯಕೀಯ ಸೇವೆಗಳು

- ಉದ್ವೇಗ ಶಿಫಾರ್ಶಿ
- ಹೃದಯದ ಉದ್ವೇಗ
- ಹೃದಯ ವಿಭಾಗ
- ಹೃದಯದ ಉದ್ವೇಗದ ಚಿಕಿತ್ಸೆ
- ವೈದ್ಯಕೀಯ ರಕ್ತಪರೀಕ್ಷೆ
- ಸೌಂದರ್ಯವರ್ಧನೆ ಚಿಕಿತ್ಸೆ
- ವಿಕಿರಣ ಚಿಕಿತ್ಸೆ
- ವಂತ ವಿಜ್ಞಾನಗಳು
- ಚರ್ಮವಿಜ್ಞಾನ
- ಮಧುಮೇಹ ವಿಭಾಗ
- ಹೃದಯಾಂಗುಲಿ
- ಹೃದಯಾಂಗುಲಿ
- ಎಂಡೋಕ್ರೈನಾಲಜಿ
- ಉದ್ವೇಗ ಚಿಕಿತ್ಸೆ
- ಉದ್ವೇಗದ ವಿಭಾಗ (ವೈದ್ಯಕೀಯ)
- ಉದ್ವೇಗದ ಚಿಕಿತ್ಸೆ

- ಸಾಮಾನ್ಯ ವೈದ್ಯ ಶಿಫಾರ್ಶಿ
- ಸಾಮಾನ್ಯ ಚಿಕಿತ್ಸೆ
- ಹಿತ್ತಲನೋವು-ಮೇದೋಜಕರಣಾಂಗ
- ಹಿತ್ತಲನೋವು ಚಿಕಿತ್ಸೆ
- ಇಂಟೆನ್ಸಿವ್ ಕೇರ್ ರೀಡಿಯಾಲ್
- ಮೂತ್ರಕುಂಡ ಕಠಿ
- ರಕ್ತ ಕಠಿ
- ಸನಾಹಿತ ಸಿಬ್ಬಂದಿ
- ಮೂತ್ರಕುಂಡ ವಿಜ್ಞಾನ
- ನರ ವಿಜ್ಞಾನ
- ನರ ಚಿಕಿತ್ಸೆ
- ಪ್ರಸೂತಿ ಮತ್ತು ಸ್ತ್ರೀರೋಗಿ ವಿಭಾಗ
- ಅಂಗಾಂಗಾಂಗ (ಗ್ರಂಥಿಪ್ರಾಪ್ತಿ)
- ವೈದ್ಯಕೀಯ ಅಂಗಾಂಗಾಂಗ
- ಚಿಕಿತ್ಸೆಯ ಅಂಗಾಂಗಾಂಗ
- ನೇತ್ರ ವಿಭಾಗ

- ಸಂಧಿ ಬದಲಾವಣೆ ಸಹಿತ ಮೂಳೆ ಚಿಕಿತ್ಸೆ ಶಿಫಾರ್ಶಿ
- ಅಡೋಪ್ಷನ್/ರೋಗಾಂಗಾಂಗಾಂಗ (ಇ.ಎಸ್.ಒ.)
- ವೇದನಾ ಚಿಕಿತ್ಸೆ
- ಮಕ್ಕಳ ವಿಭಾಗ
- ಮಕ್ಕಳ ಚಿಕಿತ್ಸೆ
- ಭೌತಿಕ ಚಿಕಿತ್ಸೆ ಮತ್ತು ಮನಃವಿಜ್ಞಾನ
- ಇನ್ಟೆನ್ಸಿವ್ ಮತ್ತು ಮನಃವಿಜ್ಞಾನ ನೋವು
- ಮನಃವಿಜ್ಞಾನ
- ಪ್ರಸೂತಿ ಚಿಕಿತ್ಸೆ (ಐ.ವಿ.ಎಸ್. ಸಹಿತ)
- ಹೃದಯಾಂಗುಲಿ ವಿಭಾಗ (ಪಲ್ಮನಾಲಜಿ)
- ಸಂಧಿಬಾಹು ರೋಗಿ ವಿಭಾಗ
- ನೆನಪುಮೂಳೆ ಚಿಕಿತ್ಸೆ
- ತೀವ್ರ ವೈದ್ಯಕೀಯ ಮತ್ತು ಸಂಧಿಬಾಹು ಚಿಕಿತ್ಸೆ
- ಮೂತ್ರಪ್ರಾಪ್ತಿ ವಿಭಾಗ
- ನಾಳಗಳ ಚಿಕಿತ್ಸೆ

ಪ್ರಯೋಗಾಲಯ ಸೇವೆಗಳು

- ವೈದ್ಯಕೀಯ ಚಿಕಿತ್ಸೆಯ ವಿಭಾಗ
- ವೈದ್ಯಕೀಯ ಸೂಕ್ಷ್ಮಜೀವಿ ವಿಜ್ಞಾನ ಮತ್ತು ಸೀರಮ್ ಪರೀಕ್ಷೆ
- ವೈದ್ಯಕೀಯ ರೋಗಿವಿಜ್ಞಾನ
- ರೋಗಿ ಚಿಕಿತ್ಸೆ
- ರೋಗಿ ಚಿಕಿತ್ಸೆ
- ರೋಗಿ ಚಿಕಿತ್ಸೆ
- ರೋಗಿ ಚಿಕಿತ್ಸೆ
- ರೋಗಿ ಚಿಕಿತ್ಸೆ

ರಕ್ತ ವರ್ಗಾವಣೆ ಸೇವೆಗಳು

- ರಕ್ತ ನಿರ್ದಿ ಮತ್ತು ವರ್ಗಾವಣೆ ಸೇವೆಗಳು

ವೈದ್ಯಕೀಯಕ್ಕೆ ಮಾರ್ಗದರ್ಶಿ ವೈಜ್ಞಾನಗಳು

- ಪದ್ಧತಿ ಅಪರಂಜಿತ ವಿಭಾಗ
- ಚಿಕಿತ್ಸೆ ಪರೀಕ್ಷಾಕಾರ
- ಪದ್ಧತಿ ಚಿಕಿತ್ಸೆ
- ಮನಃವಿಜ್ಞಾನ
- ಮನಃವಿಜ್ಞಾನ

ರೋಗಿನಿಂದ ಸೇವೆಗಳು

- 2 ಡಿ ಪ್ರಮಾಣೀಕರಣ
- ಮೂಳೆಯ ಸಾಂದ್ರತಾ ಮಾಪನ
- ಸಿ.ಟಿ. ಸ್ಕ್ಯಾನಿಂಗ್
- ಡಿ.ಎಸ್.ಎ. ಪ್ರಯೋಗಾಲಯ
- ಇ.ಸಿ.ಟಿ.
- ಇ.ಇ.ಟಿ.
- ಹೋರ್ಟ್ ಪರೀಕ್ಷೆ
- ಲಿಥೋಟ್ರಿಪ್ಸಿ (ESWL)
- ಸ್ಪಿನರೇಟಿಂಗ್
- ಎಂಜಿಂಗ್
- ಸಿ.ಎಸ್.ಎ. ಪರೀಕ್ಷೆ
- ನಿರ್ದಿಷ್ಟ ಅಂಗಾಂಗಾಂಗ
- ಪ್ರಸೂತಿಮಾರ್ಗ
- ಪ್ರಸೂತಿಮಾರ್ಗ ಪರೀಕ್ಷೆ
- ಅಂಗಾಂಗಾಂಗ
- ಅಂಗಾಂಗಾಂಗಾಂಗ
- ಅಂಗಾಂಗಾಂಗ

ಪೆಂಟಲ ಸೇವೆಗಳು

- ಫೋಟೋಗ್ರಫಿ
- ಫೋಟೋಗ್ರಫಿ
- ಫೋಟೋಗ್ರಫಿ

ಈ ಸೇವೆಗಳು ಲಭ್ಯವಿಲ್ಲ

- ಮೂಳೆ ಮತ್ತು ಕಠಿ
- ಸುಧಾರಣಾ ವಿಭಾಗ
- ವೈದ್ಯಕೀಯ ಚಿಕಿತ್ಸೆ
- ಹೃದಯಾಂಗುಲಿ
- ಪರಮಾಣು ವೈದ್ಯಕೀಯ
- ವೈದ್ಯಕೀಯಾಂಗುಲಿ ಚಿಕಿತ್ಸೆ
- ಮನಃವಿಜ್ಞಾನ
- ಮನಃವಿಜ್ಞಾನ
- ರೋಗಿ ಚಿಕಿತ್ಸೆ

2. Is there any written guidance for registering and admitting the patients?

- ✓ Yes. Sakra prepares documents detailing the mechanism for registration and admission of the patients, which includes unidentified patients & additional documentation for foreign nationals passport, visa & FRRO as required.

3. What is UHID for patient?

- ✓ Unique Hospital Identification Number. Generated at first patient-Sakra interaction.

Patient will have only one unique number. Different number will be generated for multiple visits (IP/OP) both numbers should be linked to ensure continuity of care.

4. Does Sakra summary of the patient's condition & treatment given?

- ✓ Yes. Clinical findings & treatment given are mentioned in transfer summary & discharge summary. The Sakra retains the copy of transfer/ discharge summary.

5. Who will perform the Initial assessment?

- ✓ Caregivers within their scope of practice, registration, applicable laws & regulations will perform the initial assessment. (Doctors, Nurses and Clinical nutritionist) in standardised format.

6. What is the benchmark to complete the Initial assessment of the patient by doctor?

- ✓ The maximum time limit for IP is 24 hrs.

7. What is the time limit for clinician to counter signed on care plan?

- ✓ Treating doctor/ Clinician in-charge should countersign & authorize care plan within 24 hrs.

8. Who should be reassessed & when should it be done & who should perform reassessment?

- ✓ All patients, day care patients, patients awaiting admission/bed. It is done periodically (based on setting and patient's condition) at least once daily. Treatment doctor or doctor from treatment team will do the reassessment & modify care plan if required. Nursing reassessment is done thrice per day in ward area.

9. Explain about laboratory safety programme?

- ✓ The laboratory safety programme is based on occupational health and safety management system. Laboratory safety manual ensures safety of workforce & equipment, it is in consonance with identified risks and hazards, it incorporates material safety & data sheet (MSDS), Laboratory safety programme is aligned with Sakra's safety programme.

10. How is critical results of imaging area communicated?

- ✓ Critical results will be intimated immediately to the personnel concerned. It is communicated (both in-house and outsourced results) to the member of the treating team within one hour of test completion/report being ready.

11. Is it necessary to screen the patient before imaging?

- ✓ Patient are screened for Safety/ Risk before imaging.

Patient	What should they be screened for?
Of child-bearing age	Pregnancy
Undergoing MRI	Magnetic substance

Informed consent should be taken for: Contrast injection, Moderate-deep sedation, Interventional procedures.

12. Which PPE are used for radiation safety?

- ✓ Shield body parts using lead aprons and shields. Staff directly working with radiation sources should possess and use Thermo luminescent dosimeters (TLD) badges.

13. How will we ensure adequate clinical intervention has taken place in response to a critical value alert?

- ✓ For Inpatient – Attending clinician will intervene, document the action taken in progress notes/medication orders.
For Outpatient- Patient/Family should be alerted.

14. What is an MLC?

- ✓ A Medico-Legal Case can be defined as a case of injury or ailment, etc., in which investigations by the law-enforcing agencies are essential to fix the responsibility regarding the causation of the injury or ailment example RTA, snake bite. The Sakra emergency team inform police for medico-legal cases (MLC).

15. What is LAMA?

- ✓ Patient Leaving Against Medical Advice. Treating doctor explains the consequences of action to the patient/relative.

16.Does Sakra give discharge summary to all patients?

- ✓ Yes for all patients including LAMA cases. Sakra hands over discharge summary & all` reports to the patient/relative and a copy is retained in medical record.

17.What is planed discharge?

- ✓ Discharge is planned at least 24 hrs in advance. Planning should include: Preparation of draft discharge Summary, Refund of medications. Patient education on continued care.

18.What is defined timeframe for discharge?

The start point is the time the treating doctor declares that the patient is fit to be discharged & the end point is time the patient vacates the bed. For cash patient it is 1.5 hours & for insurance patient it is 3 hours.

19.What is the contents of discharge summary?

- ✓ Patient name, UHID, diagnosis of the patient, Name of treating doctor, Name of other consultant's involved in treatment, Date of admission, Date of Discharge, Date of surgery(in case of surgery), Cause of death (In case of death) Investigation details, medication administration, Follow up advice, medication & other instructions including details of when patient can obtain urgent care.

CARE OF PATIENTS (COP)



1. Which are the two identifiers used in identifying patient?

- ✓ Patients are identified using two identifiers that is Name and UHID in the hospital

2. Do we have a Telemedicine facility at Sakra World Hospital?

- ✓ Yes

3. Which are the cases labelled as Medico Legal Case (MLC)?

- ✓ The following are some of the examples of MLCs and medical officers should use their professional judgment to decide any other cases not enumerated in the list:
 - (a) Assault and battery, including domestic violence and child abuse
 - (b) Accidents like Road Traffic Accidents (RTA), industrial accidents etc.
 - (c) Cases of trauma with suspicion of foul play
 - (d) Electrical injuries
 - (e) Poisoning, Alcohol Intoxication

4. Which is the Triage system used in Emergency for Initiation of care?

- ✓ Australasian triage scale: descriptors for categories.

TRIAGE CATEGORIES			
Number	Name	Colour	Max time
1	Immediate resuscitation	Red	0 minutes
2	Very urgent	Orange	10 minutes
3	Urgent	Yellow	60 minutes
4	Standard	Green	120 minutes
5	Non-urgent	Blue	240 minutes

✓

5. What is Emergency holding policy for the patients?

- ✓ Patients can be kept in observation room for 4 to 6 hours, after that patient is shifted out as per the patient's clinical conditions.

6. What is Color coding for triaging of the patients in Emergency in case of Disaster?

- ✓ Prioritizing all the emergency patients as per the severity of their condition into four colored zones (Red, Yellow, Green & Black) so that appropriate care can be initiated within time.

How and where do you give chest compression?

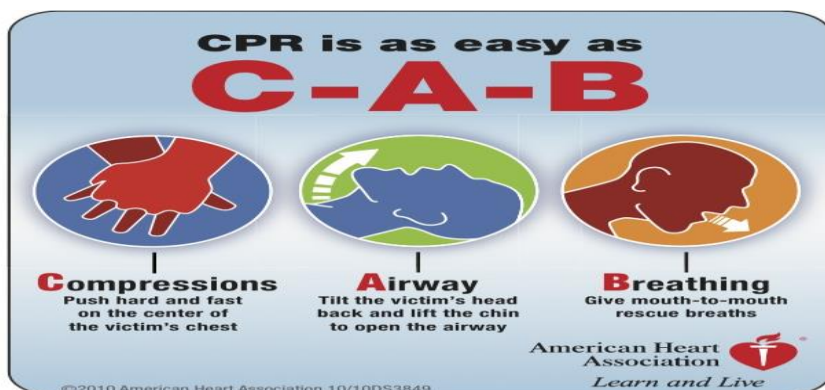
- ✓ Perpendicular to the body axis
- ✓ Point of compression- lower 3rd of sternum 4 cm above xiphoid process
- ✓ Depth of compression- at least 2 inches.
- ✓ 100-120 compression/min has to be given to patient.

7. What is the ratio of compression & ventilation?

- ✓ (30:2) 30 compressions: 2 breaths in Adults
- ✓ (15:2) 15 compressions: 2 breaths in Child and Infants

8. What is the sequence of CPR / BLS?

- ✓ CAB
 - C – Chest compressions
 - A – Airway
 - B – Breathing



9. What is Code Blue?

- ✓ Code blue means that there is an urgent medical emergency. This is usually a patient in cardiac or respiratory arrest.

10. How to activate Code Blue?

- ✓ Dial 5555 to activate
- ✓ Say “code blue” <location name> & repeat 3 times

11. What are the components of nursing care plan?

- ✓ Assessment
- ✓ Plan of care
- ✓ Implementation of care
- ✓ Evaluation
- ✓ Modification of plan of care as may be required

12. What are the measures taken to prevent Wrong patient, wrong procedure and wrong site?

- ✓ Two identifiers are used to identify patient
- ✓ Usage of Surgical safety checklist

Surgical Safety Checklist		
World Health Organization Patient Safety		
Before induction of anaesthesia (with at least nurse and anaesthetist)	Before skin incision (with nurse, anaesthetist and surgeon)	Before patient leaves operating room (with nurse, anaesthetist and surgeon)
Has the patient confirmed his/her identity, site, procedure, and consent? ☐ Yes Is the site marked? ☐ Yes ☐ Not applicable Is the anaesthesia machine and medication check complete? ☐ Yes Is the pulse oximeter on the patient and functioning? ☐ Yes Does the patient have a: Known allergy? ☐ No ☐ Yes Difficult airway or aspiration risk? ☐ No ☐ Yes, and equipment/assistance available Risk of >500ml blood loss (Yes/No in children)? ☐ No ☐ Yes, and two IV/central access and fluids planned	Confirm all team members have introduced themselves by name and role. Confirm the patient's name, procedure, and where the incision will be made. Has antibiotic prophylaxis been given within the last 60 minutes? ☐ Yes ☐ Not applicable Anticipated Critical Events To Surgeon: ☐ What are the critical or non-routine steps? ☐ How long will the case take? ☐ What is the anticipated blood loss? To Anaesthetist: ☐ Are there any patient-specific concerns? To Nursing Team: ☐ Has sterility (including indicator results) been confirmed? ☐ Are there equipment issues or any concerns? Is essential imaging displayed? ☐ Yes ☐ Not applicable	Nurse Verbally Confirms: ☐ The name of the procedure ☐ Completion of instrument, sponge and needle counts ☐ Specimen labelling (read specimen labels aloud, including patient name) ☐ Whether there are any equipment problems to be addressed To Surgeon, Anaesthetist and Nurse: ☐ What are the key concerns for recovery and management of this patient?

This checklist is not intended to be comprehensive. Additions and modifications to fit local practice are encouraged.

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13.What is DNR?

- ✓ A do-not-resuscitate order, or DNR order, is a medical order written by a doctor. It instructs health care providers not to do cardiopulmonary resuscitation (CPR) if a patient's breathing stops or if the patient's heart stops beating.

14.Do we follow do not resuscitate (DNR)?

- ✓ No

15.What is an informed Consent?

- ✓ Informed consent is a process for getting permission before conducting a healthcare intervention on a person, for conducting some form of research on a person, or for disclosing a person's information.
- ✓ Informed consent shall include documented information regarding the procedure, its risks, benefits, alternatives, and as to who will perform the procedure in a language that they can understand.

16.How surgical site marking is done? Who is supposed to do?

- ✓ Surgical site marking is done using only arrows in all cases where we need to denote laterality, digit or level.
- ✓ Doctor

17.Do we have Organ Transplant program in Sakra World hospital?

- ✓ Yes, we have Liver Transplant and Kidney Transplant.

18.What are the special cares for children taken in Pediatric OPD area?

- ✓ The environment is child friendly with playroom and breast feeding room

19.How do you manage patient fall?

- ✓ Patients need to be assessed and reassessed by nurse about the risk of fall. Morse Falls Scale is used to assess falls:
 - Score <25 Low risk
 - Score 25-45: Moderate risk
 - Score >45: High risk.

- Yellow ID band to be tied for the patient and the purpose to be communicated with the relatives
- All precaution must be taken to safeguard the patient like, side rails of bed to be kept up when no one is around, bed to be kept low with brake in position.
- Wheel chair, trolley to have strap on when transporting patients
- Caution board to be placed when cleaning is in progress to prevent slip or falls.
- Vulnerable or patients at risky must be accompanied by the attender / nurse during hospital stay.
- Patient and patient's family education is given at the time of admission after assessment and reassessment by assigned nurse, even the score is low.

20.What are patient restraint & its type?

- ✓ Restricting patient's movements with minimum discomfort and pain to prevent patients from injuring themselves and others.
- ✓ Any restraints must be humanly and professionally administered.
- ✓ Types: Physical, Chemical.

21.When is patient restraint done & does it require consent? How long it is valid?

- ✓ Clinical team treating the patient decides the need of restraints for the patient. Doctor's order must be documented in the patient's document.
- ✓ Yes, it requires consent from patient's relative / bystander. It is valid only for 24 hrs.

22.For how long is the patient restrained & when it should be reviewed? How often reassessment to be done?

- ✓ Restraints orders are valid for 24 hours only. Post that a new order must be written by the treating clinical team on the patients' document.
- ✓ Review is done after every 24 hours.
- ✓ Reassessment to be done every 2nd hourly.

23. Who are “Vulnerable Patients” in Sakra World Hospital? And how they are identified?

- ✓ Patients who are completely incapable of doing anything for themselves and require others help to eat, clean, walk etc., are recognized as vulnerable patients.

Example:

- Patients are on ventilators.
- Geriatric patients above 65 yrs.
- Pediatric patients below 12 years.
- Physically and mentally challenged patients, Etc.

24. What is Code Yellow? How and who will activate Code yellow (Community emergency/disaster)?

- ✓ A hospital's public address system alerting the staff about, and the need to prepare for, an internal or external disaster—e.g. multi trauma, major effects of storm, etc. Any CODE YELLOW announcement shall be made by the emergency room physician only after receiving authorization from the HOD-EM, Chief of Medical service (CMS) or Chief operating Officer (COO). If they are unavailable, any one of the Hospital Command Control (HCC) members can also authorize. All relevant departments of the hospital are alerted by an announcement.
- ✓ Emergency room physician calls 5555, .Advises declaration of code yellow through Public announcement system

25. What is code pink? How to activate it?

- ✓ Hospital message announced over a hospital's public address system warning the staff of an infant abduction or missing patient.

26. How to activate Code Pink?

- ✓ Dial 5555 to activate
- ✓ Say “code pink ”<location name>& repeat 3 times

27. What is end of life care?

It helps all patients those with an advance, progressive, incurable illness to live as well as possible until they die. It enables the supportive and palliative care needs of both patient and family to be identified and met throughout the last phase of life and into bereavement. It includes management of pain and other symptoms and provision of psychological, social, spiritual and practical support. It is followed at Sakra.



MANAGEMENT OF MEDICATION (MOM)

1. Who guides the formation and implementation of pharmacy services & medication usage?

- ✓ The pharmacy & therapeutic committee (PTC) consisting of Clinical pharmacology, Pharmacy, Clinicians, nursing & quality team members guide the same.

2. Where do we find Sakra drug formulary for reference?

- ✓ The Sakra portal (Intranet) contains updated drug formulary including approved list of implants. The formulary is updated on yearly basis.

3. What is drug/medication recall?

- ✓ It is an action taken to withdraw /remove the drugs from distribution or use including corrective action for which deficiencies are reported in quality, efficacy or safety based on communication from regulatory authority, manufacturer or internal feedback.

4. What is adverse events?

- ✓ An injury related to medication management, in contrast to complication of the disease.

5. What is the role of pharmacy in charge in case of recall of drug/adverse events associated with medication usage?

- ✓ He/she will inform relevant staff within 24 hours of recall/adverse events.

6. Which method of sound inventory control practice guide for storage of medication is followed by Sakra?

- ✓ First Expiry First out (FEFO) method i.e. The drugs closest to expiration in inventory are dispensed first is followed at Sakra.

7. Which are high risk /high alert medications?

- ✓ They are medications involved in a high % of medication errors or sentinel events and medications that carry a high risk for above error or other adverse outcomes. Examples: Controlled substances, psychotherapeutic medicines. They are identified by red dot marked on its package.

8. What are the important policies followed for narcotic medication management?

- ✓ **Who can prescribe:** Authorized clinician in self carbonated prescription form.
- ✓ **Who can indent?** Senior Nurse based on prescription through HIS system.
- ✓ **Who & how narcotics transported?** By nurse/technician after producing proper identification to IP pharmacy & carried in close container narcotic box.
- ✓ **Where narcotics stored?** Dedicated double locked container with two independent people holding keys.
- ✓ **Who is authorized to administer?** Doctor or nurse based on medication chart & documented with name, date, time and signature in drug chart.
What is done for unused narcotics medicine ampule or used ampule with residual? Unused sealed ampule vial is returned back to pharmacy & for residual or left out in ampule is disposed of in sink by two people and documented. All used ampules are returned to IP pharmacy with copy of prescription.
- ✓ **How is narcotics management documented?** The narcotic hardbound register both at IP pharmacy & nursing area document details based on hardcopy prescription written by clinician.

9. Where emergency medications stored in nursing area?

- ✓ Emergency medications like Atropine, Dextrose ampule, KCL are stored in emergency crash carts .The inventory of same is closely monitored at pharmacy and nursing level including stock-out of medications.

10. What is reconciliation of medications?

- ✓ It is the process of creating the most accurate list possible of all medications a patient is taking –including drug name, dosage, frequency, and route and comparing the list against the physicians admission, transfer ,and /or discharge orders, with the goal of providing correct medications to the patient at all transition points within the hospital.

11. What should be part of medication order written by doctor in medication record?

- ✓ Medicine name, route of administration, strength to be administered, frequency/time of administration, doctor's name, identifiable sign & order should be written in capital letters.

12. What is LASA medication?

- ✓ Look Alike Sound Alike (LASA) medications involve medications that are visually similar in physical appearance or packaging and names of medications that have spelling similarities and/or similar phonetics.

Look Alike medications are kept in Sakra with **Green Color border cupboard.**

Example of Look Alike medicines:

Tab. ATIVAN 1MG (LORAZEPAM) Tab. WYSOLONE 5MG (PREDNISOLONE)

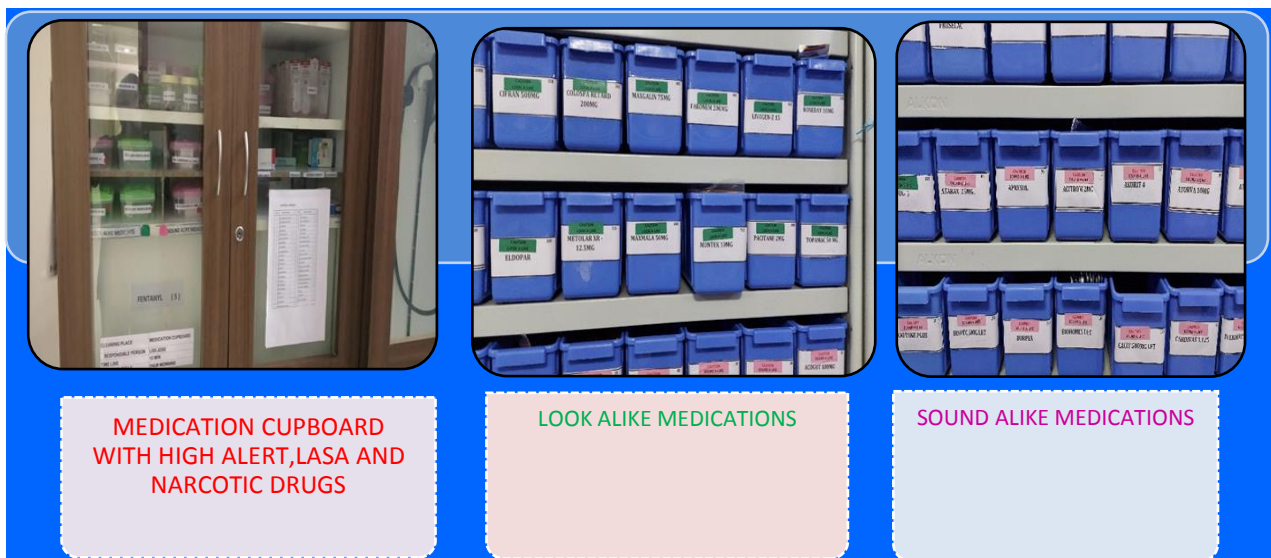
Tab. AVIL-25 MG-(PHENIRAMINE) Tab. LASIX 40MG-(FRUSEMIDE)

Sound Alike medications are kept in Sakra with **Pink Color border cupboard.**

Example of Sound Alike medicines:

Tab. ZIMIG 250MG (TERBINAFINE) Cap. Zevit (VITAMIN B.COMPLEX +ZINC)

Tab. URISPAS 200MG (FLAVOXATE) Cap. URIFAST (NITROFURATIN)





CRASH CART

- a. TOP of the cart -Defib with Accessories
- b. 1st Drawer-Airway management
- c. 2nd Drawer-Primary & Secondary drugs.
- d. 3rd Drawer-Consumables
- e. 4th Drawer- IV fluids
- f. 5th Drawer-Miscellaneous

13. What are the various information OP pharmacy should include in label while dispensing cut piece strip medications?

- ✓ Drug name, Strength, dosage instruction & expiry date are to be mentioned on label.

14. What are the checks carried out before administration of medication to patient by nurse/doctor?

- ✓ General appearance of medication, drug name, Strength, route, frequency, time are checked before administration of medication. For high risk medication, the verification of medication is done by two staff independently.

15. What is near expiry medication policy of Sakra?

- ✓ Near expiry stock of all pharmacy item will be sent three month in advance to IP pharmacy for return to vendor.

16. Which are the 6 rights of medication administration?

- 1 - Right Patient
- 2 - Right Drug
- 3 - Right Reason
- 4 - Right Dose
- 5 - Right Route
- 6 - Right Time

17. What is the policy on storage of medication at Sakra?

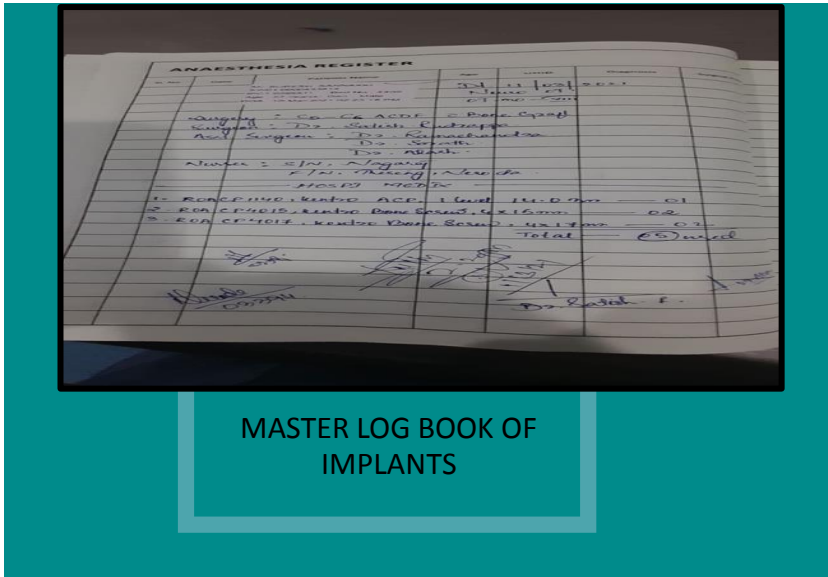
- ✓ The manufacturer's requirements are adhered to like proper temperature, clean environment. The access to medication is provided to only authorized members including locking of medication carts. The special precautions are taken for vaccine storage like medical grade refrigerator. The temperature of medication storage, refrigerator are regularly monitored and documented. The pharmacy team /nursing /quality team conduct regular audits to storage location and do stock verification and **act** based on findings. Beyond expiry date drugs are kept separately in Pharmacy (away from circulation).

18. What precautions are taken for chemotherapy medication management?

- ✓ **Prescription:** Only by authorized doctor can write who has been trained & achieved competency in monitoring and treating adverse effects of chemotherapy like medical oncologist.
- ✓ **Accessibility:** It is kept separate from other medication and only authorized pharmacists can access these medications.
- ✓ **Preparation & administering:** The qualified pharmacist/nurse with special training on preparation. The preparation /mixing of chemo drugs is done in biosafety cabinet with appropriate PPE like donning apron, mask, goggles, gloves.
- ✓ **Disposal of Chemo drug:** As per BMW policy in yellow cover bin with proper labelling.

19. What details are to be noted on implants used for patients?

- ✓ In the master log book maintained by Operation Theatre or Cath lab area, it should identify the implants used by manufacture, type, size, batch number, serial number and any other important details.



20. Who guides the formation and implementation of pharmacy services & medication usage?

- ✓ **REF: MOM1.b. -The Pharmacy & Therapeutic Committee (PTC)** guides usage of medications. This is a multidisciplinary committee with representative from various major clinical specialties, a Clinical Pharmacologist/Pharmacist, representative from nurses and Quality department.

21. Where do we find Sakra drug formulary for reference?

- ✓ **REF: MOM 2.a.** - An updated medication/drug formulary prepared by PTC a multidisciplinary committee is available at the Sakra portal (Intranet) which also has approved list of implants/devices. The formulary is updated on yearly basis.

22. What is a medication adverse event?

- ✓ **REF: PSQ 7.f.** - An injury related to medication management, in contrast to complication of the disease. Also read – Near miss/Sentinel event

23.What is the role of pharmacy in charge in case of recall of drug/adverse events associated with medication usage?

- ✓ **REF: MOM 6.b.** - He/she will inform relevant staff within 24 hours of recall/adverse events and shall be documented.

24.Which method of sound inventory control practice guide for storage of medication is followed by Sakra?

- ✓ **REF: MOM 3.b.** - First Expiry First out (FEFO) i.e. The drugs closest to expiration in inventory are dispensed first and alphabetical order is followed at Sakra.

25.Which are high risk/high alert medications?

- ✓ **REF: MOM 3.c.** - The medications with heightened risk of causing adverse outcomes and significant harm when used even with minor error. Examples: Controlled substances, psychotherapeutic medicines, LASA, concentrated electrolytes. They are identified by red dot marked on its package.

26.What is the policy on verbal orders?

- ✓ **REF: MOM 4.e.** - Verbal orders are those orders given by a physician with prescriptive authority to a licensed person who is authorized by the Sakra. It should be limited to urgent situations, documented in drug chart by nurse include repeat/read back practice to reconfirm name, strength, route etc. and countersigned by doctor who ordered it within completion of shift by clinician 24hrs. List of verbal order medicines are listed in drug formulary.

27. What is reconciliation of medications?

- ✓ **REF: MOM 4.h.** - It is the process of creating the most accurate documented list possible of all medications a patient is taking to include - drug name, dosage, frequency, and route and comparing the list against the physicians admission, transfer and /or discharge orders, with the goal of providing correct medications to the patient at all transition points within the hospital. To be communicated during handover.

28. What should be part of medication order written by doctor in medication record?

- ✓ **REF: MOM 4.b.** - all medication orders [IPD, OPD & emergency] should have - name of the patient, UHID, ascertain allergy/adverse reaction to medication, medication name in generic and capital letters, strength to be administered, frequency/time and duration of administration, route of administration, total quantity, doctor's name, signature and registration number.

29. What is adverse drug reaction?

- ✓ **REF: MOM 4.c.d.e.f.** - A response to a drug which is noxious and unintended and which occurs at doses normally used in man for prophylaxis, diagnosis, or therapy of disease or for the modification of physiologic function. The ADR is reported by filling ADR form & handed over to Clinical Pharmacology for doing root cause analysis, suggest additional CAPA if any and submit the review reports to respective consultants as well as their team with supportive evidence from literature. Reports are also submitted regularly to - Indian Pharmacopoeia Commission [IPC], as PV data base to obtain international code.
- ✓ **Examples:** Patient experiencing anaphylaxis/itching/rashes etc. soon after receiving/taking a medication.

30. What information should OP pharmacy include in the label while dispensing medications as cut strip /from bulk containers?

- ✓ **REF: MOM 6.d.** - Drug name, strength, dosage instruction in a manner that patient understands it along with expiry date to be mentioned on label.

31. What is medication error?

- ✓ **REF: US FDA [23rd Aug 2019]** – “any preventable event that may cause or lead to inappropriate medication use or patient harm while the medication is in the control of healthcare professional, patient or consumers”.

32. What precautions are taken for chemotherapy medication management?

- ❖ **Prescription:** Prescribed only by authorized doctor who has been trained & has achieved competency in monitoring and treating adverse effects of chemotherapy like a medical oncologist.
- ❖ **Accessibility:** These medications are kept separately from other medications and only authorized pharmacists can access them.
- ❖ **Preparation & administering:** The preparation is done by qualified pharmacist/nurse with special training in mixing of chemo drugs. A biosafety cabinet is used with appropriate PPE like donning apron, mask, goggle, gloves.



Dr Georgeena(clinical
pharamcologist)



mixing Chemo drugs

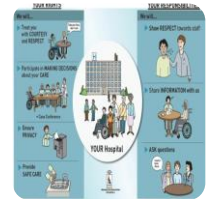
- ❖ **Disposal of Chemo drugs:** Is done as per BMW policy in yellow cover bin with proper labelling.

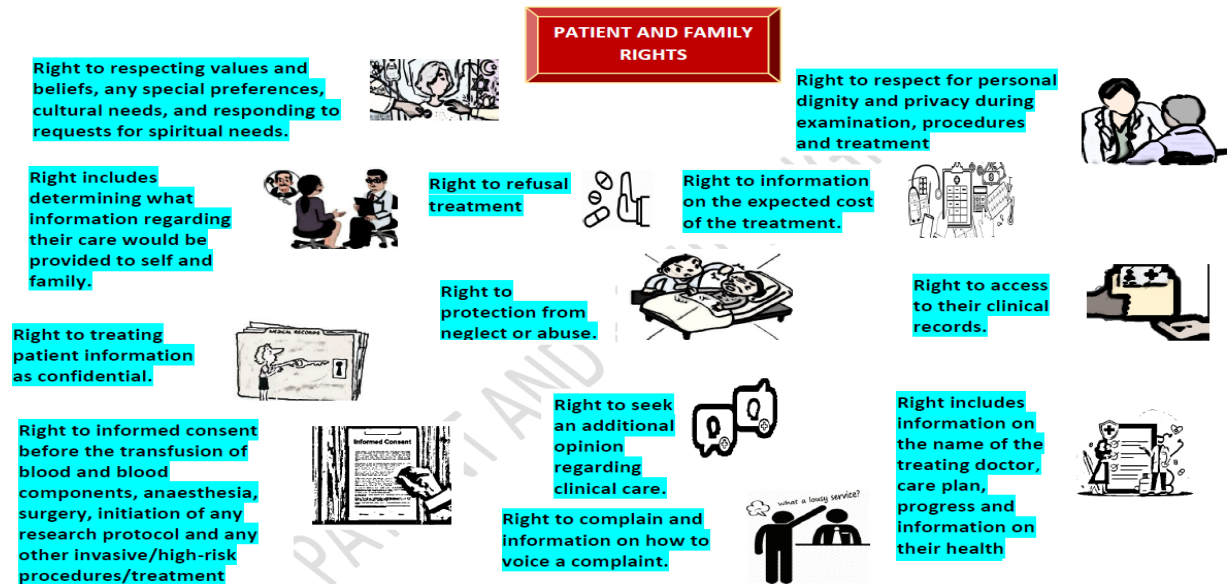
PATIENT RIGHTS AND EDUCATION (PRE)

1. What are patient and family rights and responsibilities?

Patient and family rights:

- Right to respecting **values** and beliefs, any special preferences, cultural needs, and responding to requests for spiritual needs.
- Right to respect for personal dignity and privacy during examination, procedures and treatment.
- Right to protection from neglect or abuse.
- Right to treating patient information as confidential.
- Right to refusal of treatment.
- Right to seek an additional opinion regarding clinical care.
- Right to informed consent before the transfusion of blood and blood components, anaesthesia, surgery, initiation of any research protocol and any other invasive/high-risk procedures/treatment
- Right to complain and information on how to voice a complaint.
- Right to information on the expected cost of the treatment.
- Right to access to their clinical records.
- Right includes information on the **name of the treating doctor**, care plan, progress and information on their health care needs.
- Right includes determining what information regarding their care would be provided to self and family.





Patient Responsibility

- To provide complete and accurate information, including full name, address, date of birth, current and past illness, medications, and other matters pertaining to your health
- To be on time for appointments. To cancel or reschedule as far in advance as possible in case of cancellation or rescheduling of appointments.
- To understand expenditure on your treatment and pay your bills in a timely manner.
- To provide complete and accurate information for insurance claims
- To follow the treatment plan recommended by your doctor and not to take any medications without the knowledge of doctor
- To ask questions and get clarifications about your treatment plan. To understand all instructions before signing the consent form.
- To respect that some other patient's medical condition may be more urgent than yours and accept that your doctor may need to attend them first
- To take care of your personal belongings.
- To abide by all hospital rules and regulations like, comply with the No-Smoking policy, comply with the hospital policy of silence in wards/ICUs, maintain visitor timings and visitor number, treat all staff, other patients and visitors with courtesy and respect, weapons are prohibited in hospital premises, not to mis-utilize any information, ID cards, Visitor passes, documents or any other items, keep hospital clean and hygienic.

To provide complete and accurate information, including full name, address, date of birth, current and past illness, medications, and other matters pertaining to your health



PATIENT AND FAMILY RESPONSIBILITIES

To understand expenditure on your treatment and pay your bills in a timely manner.



To be on time for appointments. To cancel or reschedule as far in advance as possible in case of cancellation or rescheduling of appointments.



To provide complete and accurate information for insurance claims



To follow the treatment plan recommended by your doctor and not to take any medications without the knowledge of doctor



To ask questions and get clarifications about your treatment plan. To understand all instructions before signing the consent form.



To respect that some other patient's medical condition may be more urgent than yours and accept that your doctor may need to attend them first



To abide by all hospital rules and regulations like, comply with the No-Smoking policy, comply with the hospital policy of silence in wards/ICUs, maintain visitor timings and visitor number, treat all staff, other patients and visitors with courtesy and respect, weapons are prohibited in hospital premises, not to mis-utilize any information, ID cards, Visitor passes, documents or any other items, keep hospital clean and hygienic



To take care of your personal belongings.



2. Give some examples of violation of patient and family rights.

- Failure to treat the patient with dignity or respect
- Compromising the privacy of patient
- Failure to protect patients from neglect or abuse
- Failure to educate patient on the informed consent
- Failure to treat patient information as confidential
- Disrespect to the religious and cultural needs of the patient
- Not providing access to medical records
- Refusal to treat the patient
- Deny the patient to take second opinion for the treatment
- Refusal to register the complaint

- Concealing the expected cost of treatment
- Concealing information such as name of the treating doctor, care plan, progress and information on their health care needs
- Discuss patient-related information in public places
- Reveal confidential information (including HIV status) without Patient's permission.
- Explicitly write/paste confidential information on the cover of the medical record.

3. How is violation of patient and family rights reported?

- ✓ Violation of patient and family rights can be reported through incident reporting system.

4. Are patient and/or family members educated and involved in the care planning and delivery process?

- ✓ Yes, they are educated and involved in the following:
 - Patient and/or family members are explained about the proposed care, including the risks, alternatives and benefits
 - Patient and/or family members are explained about the expected results
 - Patient and/or family members are explained about the possible complications
 - Care plan is prepared and modified in consultation with the patient and/or family member
 - Patient and/or family members are informed about the results of diagnostic tests and the diagnosis
 - Patient and/or family members are informed about any change in the patient's condition in a timely manner
 - Patient and/or family members are provided multidisciplinary counselling in cases such as long stay patients, potential organ donors and /or family, critically ill patients

- Surgeries
- Blood transfusions
- Anaesthesia
- Medical termination of pregnancy
- Organ transplant
- Radiation therapy
- Chemotherapy
- Biopsy
- HIV testing
- Dialysis
- Colonoscopy
- Endoscopy
- Angiogram

[illegible]

6. Who can give the informed consent?

- ✓ The consent shall be taken from the patient in all cases when the patient is capable of giving consent and above 18 years of age
- ✓ No one can consent on behalf of a competent adult
- ✓ In case patient is incapable of independent decision making, consent can be taken obtained from next of kin/legal guardian

7. Who can take informed consent?

- ✓ The person performing the procedure shall be responsible for the entire consent process, including providing explanation and taking the signature
- ✓ A doctor member of the team can take consent on behalf of the person performing the procedure

8. On what aspects of the healthcare needs are patient and families educated?

- ✓ Educated about the safe and effective use of medication and potential side effects of the medication
- ✓ Educated about food-drug interaction
- ✓ Educated about diet and nutrition
- ✓ Educated about immunizations
- ✓ Educated on various pain management techniques
- ✓ Educated about preventing healthcare associated infections

9. How is patient's feedback collected?

- ✓ Electronic feedback form (TAB) collection happens through Patient welfare officers. Patient Welfare Officer collects feedback at the time of round through (VOC) Voice of Customer. Feedback can be collected by Google reviews also.
- ✓

10.What is considered to be unacceptable behaviour at workplace?

- ✓ Alcohol and smoking at workplace
- ✓ Abusing a patient
- ✓ Inappropriate behaviour with women
- ✓ Employees fighting in the corridors
- ✓ Disrespect to any religion
- ✓ Any behaviour violating the patient right
- ✓ Talking bad about professional colleagues
- ✓ Talking bad about alternate approved system of medicine
- ✓ Corruption

11.What is the tool used to deliver a bad news to the patient and family?

- ✓ The Six-Step Protocol for delivering bad news is SPIKES.



Seven C's For Effective Communication

the **7 Cs**
for
**EFFECTIVE
COMMUNICATION**

- Correctness
- Clarity
- Conciseness
- Completeness
- Consideration
- Concreteness
- Courtesy

HOSPITAL INFECTION CONTROL (HIC)

1. What is the basic preventive measure to reduce the risk of infection?

- ✓ Hand hygiene

2. What is HAI? What are the types of HAI?

- ✓ Hospital Acquired Infection.

Types:

- CLABSI: Central line associated Blood stream infection
- CAUTI: Catheter associated Urinary tract infection.
- VAP: Ventilator associated pneumonia.
- SSI: Surgical site infection



3. List down the area comes under Highest Risk, High Risk, Medium Risk, Low Risk

- ✓ Highest Risk: Critical care area's (All ICU's, OT, Oncology, Dialysis etc.)
- ✓ High Risk: ER Room, Radiology, Lab, Dental, IVF, Blood bank, HDU, etc.
- ✓ Medium Risk: Out-patient care, Admission & Discharge etc.
- ✓ Low Risk: Office area, Cafeteria, Admin area, Lobby etc.

4. List down the assessment tool of Infection control department

- ✓ Bundle checklist, Hand hygiene, BMW management audit tool etc.

5. Who is Infection Control Link Nurse?

- ✓ Link between their assigned clinical area & Infection control team. Role of link nurse is to increase awareness of infection issues in the work areas & report infection related information like UTI, NSI.

6. What is infection outbreak?

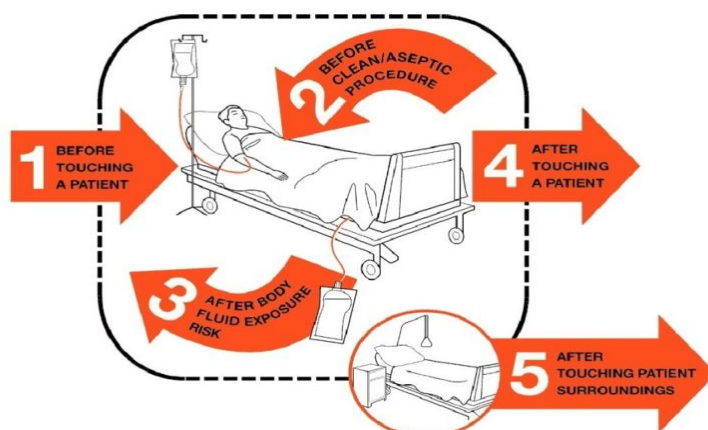
- ✓ Sudden increase in occurrence of a disease in particular time and place. Eg: Covid-19, H1N1 etc.

7. What is PPE?

- ✓ Personal protective equipment like gloves, mask, apron.



8. Hand hygiene five moments



1	BEFORE TOUCHING A PATIENT	WHEN?	Clean your hands before touching a patient when approaching him/her.
		WHY?	To protect the patient against harmful germs carried on your hands.
2	BEFORE CLEAN/ASEPTIC PROCEDURE	WHEN?	Clean your hands immediately before performing a clean/aseptic procedure.
		WHY?	To protect the patient against harmful germs, including the patient's own, from entering his/her body.
3	AFTER BODY FLUID EXPOSURE RISK	WHEN?	Clean your hands immediately after an exposure risk to body fluids (and after glove removal).
		WHY?	To protect yourself and the health-care environment from harmful patient germs.
4	AFTER TOUCHING A PATIENT	WHEN?	Clean your hands after touching a patient and her/his immediate surroundings, when leaving the patient's side.
		WHY?	To protect yourself and the health-care environment from harmful patient germs.
5	AFTER TOUCHING PATIENT SURROUNDINGS	WHEN?	Clean your hands after touching any object or furniture in the patient's immediate surroundings, when leaving – even if the patient has not been touched.
		WHY?	To protect yourself and the health-care environment from harmful patient germs.



World Health Organization

A World Alliance for Better Health Care

Patient Safety

A World Alliance for Better Health Care

SAVE LIVES

Clean Your Hands

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May 2009

9. Steps of Hand Hygiene

Steps of Hand Hygiene



01
Take adequate hand rub



02
Rub hands palm to palm



03
Right palm over left dorsum and vice versa



04
Palm to palm with fingers interlaced



05
Back of fingers to opposing palms with fingers interlaced, vice versa



06
Rotational rubbing of thumb and vice versa



07
Scour your palms with nails, vice versa



08
and your hands are safe

Do's

- Perform hand hygiene for 15 - 20 secs.
- Allow your hands to dry before wearing gloves
- Clip the nails

Don'ts

- Do not wear jewellery

10. Steps of Hand washing



11. What is Barrier nursing & in which area negative pressure isolation is available?
In which area the positive pressure isolation is available?

- ✓ The nursing of patients with infectious diseases in isolation to prevent the spread of infection.
- ✓ (cross contamination from staff to patient or Patient to staff)
- ✓ The nurses wear gowns, masks, gloves & they observe strict rules that minimize the risk of passing on infectious agents.
 - **Negative pressure area:** Bronchoscopy room & All ICU's isolation room
 - **Positive pressure area:** LTICU, OT

12. What is the mode of transmission of infection?

- ✓ Air borne- TB, Chicken pox, Herpes zoster
- ✓ Droplet – H1N1, Adenovirus, Influenza
- ✓ Contact- By touching surface area- MRSA, VRE.

13. What is Standard precaution?

- ✓ A method of infection control in which all human blood & other bodily fluids are considered infectious for HIV, HBV & other blood borne disease & regardless of patient history.

Preventive measures:

- ✓ Hand hygiene, PPE, BMW Management.

14. Safe injection and Safe infusion practices?

- ✓ One needle, one syringe, only one time.

Line policy:

- Central line – 21 days for Medicated & 14 days for Non medicated
- IV Line – ----- 72 hrs.
- Catheter – -----Silicon 21 days & Foleys 14 days
- ET/ Tracheostomy ---- As and when required.
- IV set: – -----24 hrs.
- Antibiotic----- One time use

15. What is Antimicrobial?

- ✓ An antimicrobial is an agent that kills microorganisms or stops their growth. Antimicrobial medicines can be grouped according to the microorganisms they act primarily against. For example, antibiotics are used against bacteria, and antifungals are used against fungi.

16. What is Antibiotic stewardship programme?

- ✓ Antimicrobial stewardship is a coordinated program that promotes the appropriate use of antimicrobials (including antibiotics), improves patient outcomes, reduces microbial resistance, and decreases the spread of infections caused by multidrug-resistant organisms.

17. What is the optimum spacing between two patient beds?

- ✓ 1 – 2 meter.

18. What is desired environmental condition for infection free Operating room?

- ✓ Positive pressure, 18-21 degree Centigrade, 20 Air changes per hour.

19. What precautions are taken during construction & renovation in the hospital premises?

- ✓ Any renovation & construction work in the hospital is planned with the Infection control team concerning architectural segregation, traffic flow, use of materials etc. The AC ducts leading to other area are properly sealed.

20. Which are the disinfectants used at Sakra & what is the dilution Protocol?

- Virkon – For Fogging 10 gm with 4 litre water for all areas where fogging is applicable like OT, isolation room. Surface cleaning for critical areas: 10 gm in 2 litre water in critical area like ICU, OT etc.
- Virex – 4 ml into 1 litre water for disinfection in all clinical & non clinical areas.
- Sodium Hypochlorite (4.99%): 10 ml solution for 990 ml water used for surface disinfection across hospital.
- Sodium Hypochlorite (4.99%): 100 ml solution for 900 ml water used for blood & body fluid spillage cleaning across hospital.
- R2: For surface cleaning: 20 ml in 1 litre of water.
- R5: Room freshener
- R6: WC & urinal bowl cleaning: Ready to use.
- R9: Rest room floor cleaning: 50 ml in 1 litre of water.
- R20: Polish remover: 200 ml in 1 litre of water.
- Emeral: Cream cleanser solution for descaling. Ready to use.

21.BMW SEGREGATION

Use me- Yellow

Category	Type of bag	Type of waste	Treatment/ Disposal
YELLOW	Non-chlorinated plastic bags	- Human Anatomical Waste - Expired or Discarded Medicines - Cytotoxic drugs along with glass or plastic ampoules, vials etc. - Chemical Waste - Micro, Bio-t and other clinical lab waste - Chemical Liquid Waste - Discarded linen, mattresses, beddings contaminated with blood or body fluids - Mask & surgeon apron.	Incineration or deep burial*

YELLOW

Use me- Red

Category	Type of bag	Type of waste	Treatment/ Disposal
RED	Non-chlorinated plastic bags or containers	Contaminated/ non contaminated Waste (Recyclable) Vaccination tubing bottles intravenous tubes and sets catheters, urine bags, syringes (without needles) and gloves	Autoclaving/Shredding & sent for recycling

RED

USE ME- BLUE

• All intact glass bottles and vials.

BLUE

SHARPS-PUNCTURE PROOF CONTAINER

All needles to be discarded in the Sharp container which is rigid puncture proof Container.

WHITE (Translucent)

PPC(Puncture Proof Container)

22. Explain the management of Infected Linen at Sakra?

- ✓ They are collect in white linen cover. Ward label is placed and send it to linen dept.

Linen dept. will soak it in sodium hypochlorite and send it to vendor.

23. What is the guidelines for safe food storage?

- ✓ Date of manufacture, Date of expiry & Opening Date should be mentioned. Food item storage should be done above 6 inch from floor.
 - Refrigerator temp should be monitored and recorded.
 - Cleaning reagents should be kept away from food items.
 - Preventive measures to be taken for flies & insects to keep away from food.

24. What are the preventive measures for CAUTI, VAP, CLABSI, SSI?

- ✓ Follow the respective bundle checklist for each shift by nursing team.

25. Which are various area wise zones for CSSD?

- ❖ Red zone: Receiving, cleaning and disinfection of Instruments.
- ❖ Yellow zone: Segregation, Inspection and packing, loading.
- ❖ Green zone: Sterile room, Dispatch area. The sterilisation happens between yellow & green zone.

26. What is SUD (Single use device)?

- ✓ . The term single use device means the device that is intended for one use, or on single patient during the single procedure. The labelling identifies the device as disposable & does not provide instructions for reprocessing. These are used multiple times for economic purpose, limited resources after taking patient consent.

No of re-use & process of re-use are defined & monitored by user. The marking is made on same by CSSD team.

For e.g. – Octopus used in CABG – 20 times

EVH-----8 times & Cath lab catheter – 3 times etc.

27. What is CSSD Recall process?

- ✓ In case of failure in biological indicator or water droplets found in set post sterilization, user department informs CSSD on same and all sets loaded in that particular batch of sterilization are recalled by CSSD team for reserialization.

28. Explain about NSI Follow up schedule?

- ✓ 0, 6 week, 3 months, 6 months, 1 year.

29. Give details about Staff Immunization?

- Hep-B for doctors, nurses, housekeeping including outsourced, paramedical staff & TT is additional for HK & CSSD staff.

30. What is the protocol to be followed in Needle stick injury (NSI)?

- Do not panic
- Pricked finger should not be put into mouth by reflex.
- Do not squeeze the injured site.
- Wash the area with soap and water immediately
- Cover cut injury with waterproof dressing.
- All sharps injuries incidents are emergency situation and need to be reported to an immediate supervisor and Infection Control Nurse/Officer immediately after the incident. (After hours, report Contact Nursing Supervisor immediately)
- Fill in the designated EPI net form
- Regardless of the source of exposure, the recipient should immediately be examined and the risk assessed by a physician or trained health care worker with experience in the management of blood-borne infections (HIV, Hepatitis B and Hepatitis C).
- Take HEP-B vaccination details and vaccinate if required
- Review of details of the specific event and medical history (if available) of the source patient are helpful.
- Follow up: 0 day, 6 week, 3 month, 6 month and 1 year.

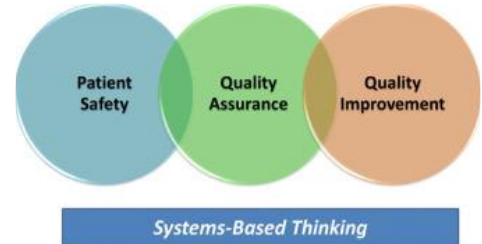
PATIENT SAFETY AND QUALITY (PSQ)

1. What is an audit?

- ✓ It is the process of reviewing of delivery of care to identify deficiencies so that they may be remedied.

2. What are the types of incidents to be reported?

- Near Miss.
- Adverse Event.
- Sentinel Event.



Incident Category

Category	Definition	Error to patient	Patient Condition	Example
A	Circumstances or events that have the capacity to cause error.	Not reached	No harm	Corridor was wet between 3A and 3B.
B	An error that did not reach the individual.	Not reached	No harm	Before using the wheelchair, it was found that the brake was not working.
C	An error that reached the individual but did not cause harm.	Reached	No harm	Wheelchair was used for patient transfer, it turned out that the belt was missing so the wheelchair was changed.
D	An error that reached the individual and required monitoring or intervention to confirm that it resulted in no harm to the patient.	Reached	No harm	Patient fell down. Patient claimed the pain and found out there was no harm.
E	Temporary harm to the individual and required intervention.	Reached	Temporary harm	Patient fell down. Patient claimed the pain and found out there was bruises on his arm and the pad was applied.
F	Temporary harm to the individual and required initial or prolonged hospitalization.	Reached	Temporary harm	Patient fell down. Patient claimed the pain and found out the arm was broken. Patient underwent the surgery.
G	Permanent harm to individual.	Reached	Permanently harm	Patient fell down. Patient condition was checked and turned out patient won't be able to walk anymore.
H	Intervention required to sustain life.	Reached	Permanently harm	Patient fell down and hit the head on the floor. Patient got brain damage and led to coma.
I	Death of individual.	Reached	Dead	Patient fell down and hit the head on the floor. Patient died.

3. What is the meaning of "Near Miss"?

- ✓ Any process variation that did not affect an outcome but for which a recurrence carries a significant chance of a serious adverse outcome Examples: 5% dextrose about to be given to diabetic patient but was averted.

4. What are sentinel events?

- ✓ A **sentinel event** is defined as any unanticipated event in a healthcare setting resulting in death or serious physical or psychological injury to a patient or patients, not related to the natural course of the patient's illness.

5. How to report sentinel events?



- ✓ By filling Online Incident Report.

6. What are Incident Reports?

- ✓ An incident report is a form that is filled out in order to record details of an unusual event that occurs at the facility, such as an injury to a patient. The purpose of the incident report is to document the exact details of the occurrence while they are fresh in the minds of those who witnessed the event.

7. Where are Incident Report forms?

- ✓ This is available on all the desk tops (Portal) in all the departments in Hospital.

8. Within what time the incident has to be raised?

- ✓ Within 24 hours of the incident occurring.

9. Within what time the RCA should be completed?

- ✓ Within 48 hours of the incident occurring. For sentinel event it is seven working days.

10. Which are the Patient Safety Goals of Sakra World Hospital?



- ❖ Goal 1: Identify patients correctly.
- ❖ Goal 2: Improve effective communication.
- ❖ Goal 3: Improve the safety of high-alert medications.
- ❖ Goal 4: Eliminate the Wrong Site.
- ❖ Goal 5: Reduce the risk of health care-associated infections.
- ❖ Goal 6: Reduce the risk of patient harm resulting from falls.
- ❖ Goal 7: Medication accuracy at transition in care
- ❖ Goal 8: Avoiding tubing and catheter misconnections

11. Do we have a patient safety and Clinical safety officers?

Yes & they take monthly safety rounds and report back to safety committee.

12. What do you know by term Hazard identification & risk analysis (HIRA)?

Hazard Identification and Risk Assessment (**HIRA**) is a process that consists of a number of sequential steps such as hazard identification, consequence & frequency assessment, risk estimation based on the existing controls and recommendations to reduce those risks which are not under acceptable limits. Each functional department have their own risk assessment HIRA table & same is reviewed on yearly basis or during any unforeseen incidents noticed.

13. What is Kaizen?

Kaizen is a Japanese term meaning "change for the better" or "continuous improvement." It is a Japanese business philosophy regarding the processes that continuously improve operations and involve all employees. Kaizen sees improvement in productivity as a gradual and methodical process.

Key performance indicators (KPI) attend analysis helps team to take up new Kaizen to achieve acceptable quality level/benchmarks like reduction in discharge turn over time as Kaizen (Quality improvement) project.

14. What is PROM (Patient reported outcome measures)?

Patient reported outcome measures (PROMs) are questionnaires patients complete on their health and quality of life. The information collected from PROMs can help to monitor patient progress, facilitate communication between professionals and patients and/or help to improve the quality of health services.

15. Which all committee meetings are conducted in Sakra World Hospital?

- ✓ The committee meetings carried out in Sakra World Hospital are:
 - a. Mortality and Morbidity committee
 - b. Infection control committee
 - c. Quality steering Core Committee
 - d. Patient Safety Committee
 - e. Blood Transfusion Committee
 - f. Pharmacy and Therapeutic Committee
 - g. CPR committee
 - h. Ethics committee
 - i. Medical records department committee
 - j. Sentinel event committee
 - k. Internal compliant committee

16. Which quality improvement tools are used as part of Kaizen (quality improvement project)?

Analytical tools: Fish bone chart, Pareto chart, flow chart, histogram, stratification

Statistical tools: Data Sampling, Control chart, ANOVA, Hypothesis analysis etc.

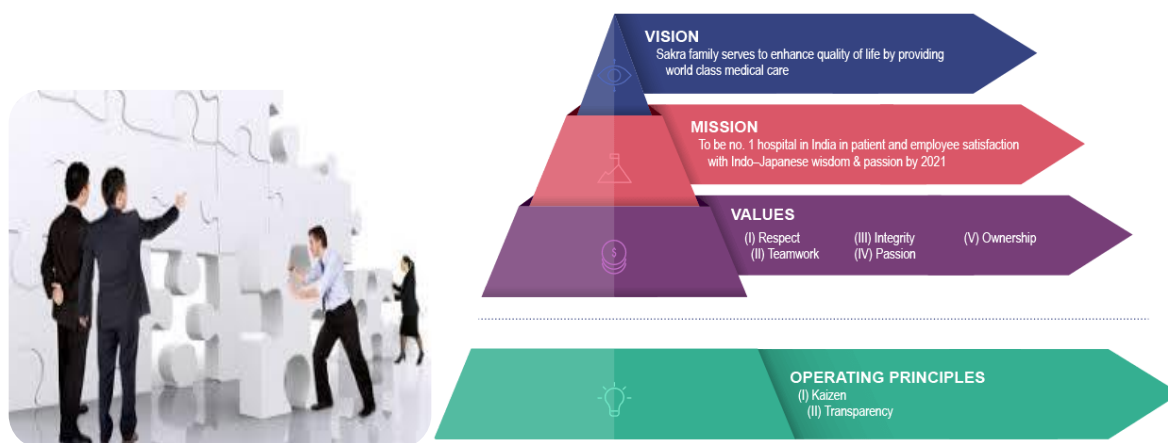
Managerial tools: Affinity diagram, matrix diagram, PDCA, Lean, Six sigma etc.

17. Which broad topics can be selected as Clinical audits?

The topics can be disease/cost / community /morbidity (Length of stay) based.

RESPONSIBILITY OF MANAGEMENT (ROM)

1. What is the vision, mission, core values and operating principles of our hospital?



2. What kind of accreditations and/or certificates we are having?

- ❖ **NABH (NATIONAL ACCREDITATION BOARD FOR HOSPITALS AND HEALTHCARE PROVIDERS)**
 - ✓ Sakra World Hospital is accredited by the National Accreditation Board for Hospitals & Healthcare Providers (NABH), the highest recognition for quality patient care and safety in India thereby joining the league of very few NABH accredited hospitals in Bangalore.
- ❖ **NABL (NATIONAL ACCREDITATION BOARD FOR TESTING AND CALIBRATION LABORATORIES)**
 - ✓ Ours is the first hospital based lab in Karnataka to get the NABL (National Accreditation Board for testing and calibration Laboratories) accreditation within one year of operations for the entire scope of services - Clinical Pathology, Cytopathology, and Clinical chemistry, Haematology, Immunohematology, Histopathology, Microbiology and Serology.

❖ **NABB**

- ✓ Sakra World Hospital Blood Bank is accredited by NABH for good practice
- ❖ **Nursing Excellence Certification** towards highest compliance to nursing good practice towards patient care.

3. What is used for comprehensive risk management in our hospital?

- ✓ “Hazard Identification and Risk Assessment (HIRA)” is been implemented entire our hospital.

4. What is Hazard?

- ✓ Anything that can cause harm or made you get accident such as electric, chemical, work on ladder, machine etc.

5. What is Risk?

- ✓ Chance high or low that any hazard will actually cause somebody harm and/or something damage.

6. Five steps of HIRA

- Step 1: Identify the hazards (places, works)
- Step 2: Estimate the degree of hazard about the probability and the severity of the accident
- Step 3: Evaluate the risk level
- Step 4: Study risk reduction measures from higher priorities
- Step 5: Determine risk reduction measures and implement them.

*Components of Risk (Probability x Severity)

Degree	Probability	Severity		
		To Human	To Property	To Business
5	Highly likely	Catastrophic		
4	Likely	Major		
3	Possible	Moderate		
2	Unlikely	Minor		
1	Rare	Negligible		

*Risk Assessment Matrix

Threat	Probability (Pb)	Severity (H+P+B)	Risk Rating Pb x (H+P+B)
Patient Fall	4	4+1+1	24
Bomb Attack	1	5+5+5	15
Data loss due to virus attack	3	1+1+5	28

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Risk Score	Definition	Countermeasure
-11	Low risk	Should be implemented
12-23	Moderate	Should be implemented to the near term
24-44	Very High	Should be implemented as soon as possible
45 -	Extreme	Should be implemented immediately

7. What kind of reporting systems for process failures are there in our hospital?

- 1) Internal: Escalation Matrix (in each department)
- 2) External Escalation Matrix like Police, Fire, Electrical board.

FACILITY MANAGEMENT AND SAFETY (FMS)



1. Name few examples of patient safety devices found in Sakra?

- ✓ Grab bars at patient toilets, Bed rails of patient beds, safety belts of wheel chair, nurse call bells at patient room etc.

2. Is Sakra having special toilets for differently abled (handicap) patients?

- ✓ Yes & they are found in each floor which have access to wheel chair like multi OPD, near 3rd B ward.

3. How often the facility safety round being conducted at Sakra & who are its participating members?

- ✓ It is conducted once every month .Quality executive, Facility Engineer, Security officer & housekeeping executive and invited members participate in facility inspection round.

4. What alternate electrical facility is available at Sakra in case of city power failure?

- ✓ We have Diesel generator (DG) sets to take care of entire hospital electrical load and uninterrupted power supply (UPS) for critical area like OT, ICU, Cath lab.

5. Give example of few energy efficient measures adopted by Sakra?

- ✓ We have LED lights across hospital which consume less electricity, we have solar heater to provide hot water. We recycle our used water for gardening & toilet flushing.

6. How security of patient & staff are ensured by Sakra security team?

- ✓ Security staff deployed at prominent location identify each person based on identification like ID band of patient, Identification card for employee, outsourced staff, Vendors, patient attender and visitor. Hospital has installed CCTV at various location and it is being centrally monitored .The security has installed controlled door access in sensitive area like OT, Cath lab, ICU to restrict unauthorized people movement.

7. Give example of hazardous materials found in hospital?

- ✓ Biological materials like blood, body fluids, microbiological cultures, medical gases, ETO, housekeeping chemicals etc.

8. What action is to be initiated when spillage of hazardous spillage happens within hospital?

- ✓ Code Hazmat to be announced by staff mentioning floor & location by dialing 5555 number to seek support from Hazmat team.

9. What are the contents of Spill Kit?

1. Sign board	10. Eye shield mask
2. Gum boot	11. Torch
3. PVC suit with hood.	12. Observant pad.
4. 3M respirator mask.	13. Different types of tissue paper.
5. N/95 Mask	14. Dustpan with brush
6. General Mask	15. Cardboard pieces.
7. Nitrile gloves	16. Garbage covers (All types)
8. Yellow rubber gloves	17. Cable tie
9. General gloves	

10. What is the procedure to manage blood spill?

- ❖ **STEP 1:** Wear disposable clean or industrial gloves
- ❖ **STEP 2:** Keep a caution board
- ❖ **STEP 3:** Cover the spill immediately with absorbent material like Tissue towel or news paper
- ❖ **STEP 4:** Wear all PPE (Apron, Mask with eye shield and industrial gloves). Add 10ml of sodium hypochlorite solution to 90ml of water and mix well
- ❖ **STEP 5:** Cover the area of spill with 10% sodium hypochlorite solution freshly prepared (5000 to 4000 ppm of available chlorine) and leave it for 10 minutes.
- ❖ **STEP 6:** Use the cardboard to scoop the material and discard
- ❖ **STEP 7:** Discard the contaminated waste in the yellow liner of infectious waste
- ❖ **STEP 8:** Wipe the area with absorbent material to remove if any remaining blood
- ❖ **STEP 9:** Clean the area with water and normal floor cleaning solution
- ❖ **STEP 10:** Send the mop to the laundry for washing, clean and disinfect the bucket for next use
- ❖ **STEP 11:** Wash hands

11. How do we identify any particular engineering or medical equipment?

- ✓ Each equipment is identified by unique asset number which may be found on surface of that particular equipment.

12. How do you know if medical equipment is safe and give accurate results?

- ✓ By checking if particular equipment is timely performed preventive maintenance and calibration by Clinical engineering team. We normally find on surface of equipment the identification sticker with details.

13. Where do we find communication contact details of engineering & Clinical engineering staff?

- ✓ It is found as display in each nurse station as escalation matrix. It can also be viewed in Sakra Intranet.

14. What are the important contact no. of support services?

Department	Speed Dial
Engineering	2828
Clinical Engineering	2842
Housekeeping	2850
F&B	2069, 2070
IT	3434
Security	5555

15. How do you clean the medical equipment?

- ✓ It is followed as guidelines of service providers manual or as per direction of infection control team. **Example:** Patient Physiological monitor: With 1% hypochlorite solution Screens/Display unit of medical equipment : Dry cloth

16. What are different services under MGPS (Medical Gas Piped system)?

- Oxygen
- Nitrous Oxide
- Compressed Air- 4bar
- Compressed Air- 7bar
- Vacuum

17. What are the types of medical gases used in the hospital and what are the colours of the pipes those carry them?

Medical Gases	Colour of Pipeline
Oxygen	Canary yellow with white band
Nitrous Oxide	Canary yellow with blue band
Compressed Air-4bar	Sky blue with white and black band
Compressed Air-7bar	Sky blue with black band
Vacuum	Sky blue with yellow band

Medical Gas	Colour of Cylinder	Example
Oxygen	Black body with white shoulder	
Nitrous Oxide	Persian blue	
Carbon-dioxide	Grey with silver shoulder	

18.Name radiation safety devices?

- ❖ Lead Apron
- ❖ Gonad shielding
- ❖ Thyroid shielding



19.What is a fire extinguisher?

✓ Fire extinguisher is the first firefighting equipment to be used in case of fire.

20.What to do in case of fire?

- ❖ Dial 5555 and activate Code Red
- ❖ Inform exact location
- ❖ Tell your identity (Inform twice)
- ❖ Try to extinguish the fire with the available and appropriate firefighting equipment.

21. Where is the nearest fire exit?

- ✓ There are six fire exits on all each floors (please know exits on your floor)

22. What is the location of fire extinguisher?

- ✓ All lift lobbies, all fire exit staircase etc. (please know fire extinguisher on your floor / dept.)

23. What is the Hospital's policy on smoking?

- ✓ Sakra World Hospital is a "NO SMOKING" area.

24. How to use fire extinguisher (minor fire)?

- ✓ We can use fire extinguisher by following the **PASS method**
 - ❖ **P**- Pull the pin,
 - ❖ **A** -Aim the nozzle at the base of the fire,
 - ❖ **S**-Squeeze the handle,
 - ❖ **S**- Sweep from side to side.



What would you do in case of major fire?

- ❖ Activate code RED immediately.
- ❖ Use of nearest fire hydrants as per the direction of fire safety officer & type of fire
- ❖ Evacuation process to be carried out to the assembly area

25. Class of fire.

- 1) A (Solid Fire) : Wood, paper, cloth, Plastic etc.
- 2) B (Liquid Fire) : Petrol, Diesel, Oil, Kerosene, Paint etc.
- 3) C (Gas Fire) : LPG, CNG, Nitrogen, Welding gas etc.
- 4) D (Metallic Fire) : Zinc, Potassium, Sulphate etc.
- 5) E (Electrical fire)

26. Few examples of internal disaster?

- Fire can be of two types: Major & Minor
- Bursting of autoclave machine,
- Sudden failure of electricity,
- Bursting of pipelines,
- Flooding of basements etc.

27. Few examples of non-fire emergencies?

- Child abduction/person missing
- Violent patient/attendant

HUMAN RESOURCE MANAGEMENT



1. What is the difference between job description and job specification?

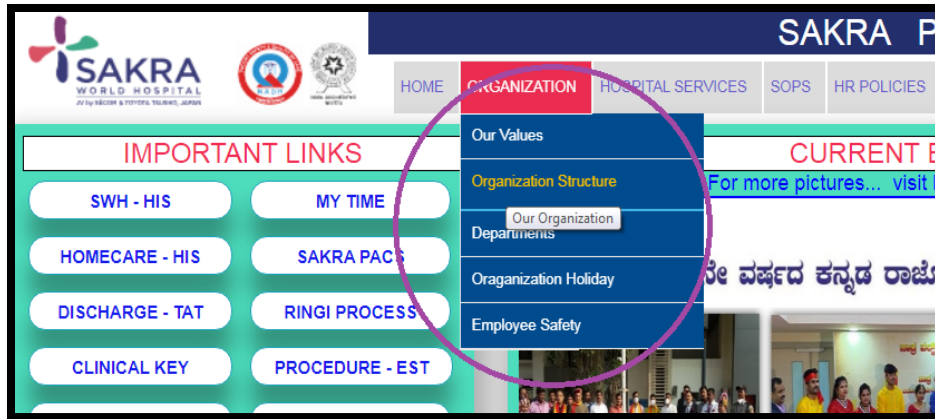
- Job description gives an explanation pertaining to duties, responsibilities and conditions required to perform a job.
- Job specification is the qualifications/physical requirements, experience and skills required to perform a particular job/task.

2. What is an organogram?

- ✓ An organogram is a graphical representation of Sakra's structure.

3. Where can you find the organogram of our hospital?

- ✓ Sakra Portal and Darwin Box



4. How does Sakra ensure that the staff is fit to provide safe care to patients?

- ✓ By conducting a pre-employment medical examination.

5. Can you mention about the few acts which are treated as MISCONDUCT at Sakra?

- ❖ An act to lower the image of the Company in the eyes of the public.
- ❖ Sabotage or damage to any property of the company.
- ❖ Gambling within the Sakra premises or Sakra provided transportation or Sakra provided Accommodation.
- ❖ Compromising with the respect of patient rights, human rights, dignity and legitimate interests of all individuals directly and indirectly associated with us.
- ❖ Neglecting the patient safety norms and protocols.
- ❖ Misuse of pharmaceutical drugs including narcotic drugs.
- ❖ Participating in any kind of fraudulent activities. Engaging in criminal conduct of any kind. Engaging in criminal conduct or act of violence or making threat of violence toward anyone on Sakra premises.
- ❖ Proxy registering of attendance or abetting in the act of registering attendance of another employee.
- ❖ Breach of any law relating to intoxicating drinks or drugs in force in any area in the working premises as well in hostel premises.
- ❖ Distribution or exhibition of any newspapers, handbills, pamphlets, etc.
- ❖ Taking or giving bribes or any illegal gratification or indulging in corrupt practices.

6. What are the topics which were taken in Sakra induction program?

- ❖ Sakra's Vision, Mission and values.
- ❖ Employee rights and responsibilities.
- ❖ Patient rights and responsibilities.
- ❖ Safety training including emergency codes.
- ❖ Basic life support.
- ❖ Hospital infection prevention and control.
- ❖ Service standards of the Sakra.
- ❖ Administrative procedures like leave, attendance, conduct etc.
- ❖ Departmental induction (at respective department)

7. How does Sakra deal with risk management?

- ✓ There is specified HIRA (hazard identification and risk assessment) format to detect, handle, minimize and eliminate the risks associated with in the Sakra 's environment. HIRA is specific to respective departments.

8. What is the frequency of performance appraisal carried out at Sakra?

- ✓ Once a year.

9. What is the mechanism/policy followed by our hospital for disciplinary and grievance handling?

- ✓ Grievance raised by any staff is handled by a formal committee and is expected to be resolved within 10 working days.
- ✓ The staff could approach the Grievance Redressal Committee with the details pertaining to the grievance, through a written complaint or via email to sakra.GRC@sakraworldhospital.com
- ✓ It can be further escalated to COO if the grievance is not resolved within the timeline or the staff is not satisfied with the resolution.
- ✓ Voice of Sakra at Portal in another mode to reach management.

10. How does Sakra ensure staff well-being?

- ❖ By conducting Annual health check for all employees,
- ❖ By promoting healthy lifestyle programs
- ❖ Having defined working hours and workload monitoring
- ❖ Stress management sessions
- ❖ Access to dining facilities
- ❖ Rewards and recognition like Kaizen.
- ❖ Staff engagement activities like singing, drawing competitions.
- ❖ Taking steps to combat occupational health hazard.

11. Which are the common training the employees have to undertake on regular basis?

- ✓ VMV, Occupation safety, Fire safety, Infection prevention, Employee Rights & responsibilities, Patient rights & responsibilities, Kaizen & quality initiatives, Incident reporting & HIRA, Disaster management.

12. What is ICC & who are its members?

- ✓ ICC stands for **internal complaint committee** and it is constituted in order to receive and address Sexual Harassment complaints.

Members:

- Head - Laboratory Medicine – Presiding officer
- Chief Nursing Officer – Committee member
- Senior Consultant- Cardiac Anesthesia - Committee member
- Manager Human Resources - Committee member
- Advocate, External committee member.

13. Give the examples of workplace related injuries?

- ✓ Needle stick injury, back injuries sustained during patient transport, hearing impairments due to noise levels, fall due to unsafe facility etc.

14. What is privileging? How often it is reviewed?

- ✓ Privileging is the process for authorizing all medical professionals to admit and treat patients and provide other clinical services commensurate with their qualifications and skills.
- ✓ Privileges granted to an employee shall be reviewed, reassessed at the time of promotion/reassignment or once in three years whichever is earlier.
- ✓ Privileging is done for nursing & paramedical staff.

15.Mention few Employee rights and Responsibilities.

EMPLOYEE RIGHTS:

- ❖ To be aware of the terms and conditions of his/her employment before joining the Sakra.
- ❖ To be aware of the hospital wide policies.
- ❖ To be entitled to the terms and conditions as specified in the appointment letter.
- ❖ To ask for his/her role clarity.
- ❖ To have a safer workplace.
- ❖ To take a meal break during the day.
- ❖ To avail the medical benefits as per the policy laid down by the company.
- ❖ To register a complaint or file a grievance by filling up a Grievance form.
- ❖ To get the salary in the first week of every month for the number of days worked for the previous month.
- ❖ To be treated considerately and respectfully, and not discriminated on the basis of caste, religion, gender or socioeconomic background.
- ❖ If any one believes that he/she has been the victim of harassment, or knows of another employee who has, the right to report it immediately to the HR department.
- ❖ To avail the benefits being extended by the Sakra to particular category of employees, if he/she is part of that category.
- ❖ To be aware of all the rights being conferred on an employee during the course of employment.
- ❖ To choose not to continue working with the Sakra by serving the notice period as applicable to him/her.

EMPLOYEE RESPONSIBILITIES:

- ❖ To arrive at work on time.
- ❖ To dress suitably for the job (uniform in case applicable for the job).
- ❖ To perform to the best of the abilities, without fear or favor.
- ❖ To obey safety rules.
- ❖ Not to discriminate or harass others in the workplace.
- ❖ Not to act in a way that puts the employee or others at risk of injury in the workplace.
- ❖ To notify supervisor / HOD prior to absence. In the event of emergencies, when prior notification is not possible, employees are obligated to notify their supervisor / HOD of their absence at the earliest possible time.

- ❖ To be familiar with the work standards associated with his/ her job performance. Standards for work performance may be obtained from the supervisor as well as the job description.
- ❖ To immediately report injuries or accidents.
- ❖ To get vaccinated as per existing protocol in the hospital.
- ❖ To complete on-going education / training appropriate to their role or as stipulated by the department concerned.
- ❖ To carry out reasonable requests and instructions of the supervisor.
- ❖ For maintaining confidentiality at workplace.
- ❖ To inform the HR Department for change in name, address, telephone number, marital status or number of dependents.
- ❖ To wear their ID card during the duty hours and while being in the hospital premises.
- ❖ To immediately report injuries or accidents.

16. What is occupational health hazard?

The hazard to which an individual is exposed during the course of the performance of their job. These include physical, chemical, biological, mechanical and psychosocial hazards.

INFORMATION MANAGEMENT SYSTEM (IMS)

1. What is time frame for patient's records to reach MRD?

- ✓ Within 24 hrs. of discharge

2. Who are allowed to make entries in medical records?

- ✓ Following are authorized to make entries in medical records:
 - Doctors
 - Nurses
 - Physiotherapists
 - Dieticians
 - MRD staff (ICD coding only)
 - Clinical Psychologist
 - Para-clinical staffs



3. What is the retention period of records?

Category	Minimum Retention Period (in Years)
Inpatient Medical Records	05 years
Pediatric In-Patient Medical Records	05 years or till child attains age of 18
Expired Cases (Inpatient)	10 years
Medico legal Cases (Inpatient)	Life time
Death Register	Permanent
Birth Register	Permanent
MLC Register	Permanent

In each of above when more than one category applies, the one with longer retention period is followed.

4. What is Notifiable Disease?

- ✓ Certain specified diseases, which are required by the law to be notified to the public health authorities are called notifiable diseases. For example,
 - Polio
 - Influenza
 - Malaria
 - Rabies
 - HIV/AIDS
 - Tuberculosis

5. What is HIS?

- ✓ Hospital information management system is a comprehensive integrated information system designed to manage all the aspects of the hospital's operation, such as medical, administrative, financial, and the corresponding processing of services.

6. How is confidentiality of patient's records maintained?

- **Access control:** Control accessibility to MRD and HIS.
- **Electronic system:** Provide different access to different personnel and specific for that user.
- **Physical records:** Use of tracer card to trace movement of files in and out of MRD.

7. What is Privileged Information?

- ✓ Confidential information furnished (to facilitate diagnosis and treatment) by a patient to a professional authorized by law to provide care and treatment is called privileged information.

8. Can privileged health information be disclosed?

- ✓ It can be disclosed only when authorized by the patient and/or as required by law, such as, in case of medico-legal cases.

QUALITY & SYSTEMS



Teamwork | Respect | Integrity | Passion | Ownership

Sakra World Hospital, 52/2 & 52/3, Deverabeesanahalli, Varthur Hobli, Bangalore 560 103.

(Landmark: Outer ring road, behind Passport office, Opp. Intel, Marathahali)

